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MINISTRY OF HEALTH-ETHIOPIA

NATIONAL FAMILY PLANNING SERVICE GUIDELINE FOURTH EDITION

2025-2030



December, 2024

FORWARDED

It is with great pride and commitment that I present this foreword for the *Short-Acting Family Panning (FP) Methods* training module. As a representative of the Ministry of Health, I am deeply aware of the pivotal role FP plays in the health and well-being of our citizens, as well as its substantial impact on the socio-economic development of our nation.

FP is more than a health intervention; it is a fundamental human right that enables individuals and couples to determine the timing and spacing of their children. By providing access to effective and safe FP methods, we empower women and men to make informed decisions about their reproductive health. This empowerment translates into healthier families, stronger communities, and a more prosperous society.

FP methods can be categorized into natural, short-acting, long-acting, and permanent methods. Each type has unique benefits and is suited to different needs and life stages. Short-acting methods include pills, injectable, and barrier methods. Pills, taken daily, are a popular and effective method for many women.

To ensure effectiveness and accessibility, several key areas must be addressed:

- Education and Awareness: Providing accurate and comprehensive information to dispel myths and empower informed choices.
- Access and Availability: Ensuring services are available to all, regardless of geography or socio-economic status, supported by a steady supply of contraceptives and trained providers.
- Quality of Care: Equipping healthcare providers with the necessary skills and knowledge through continuous training and support.
- Supportive Policies: Securing sustained funding, resources, and partnerships with both international organizations and local communities.

As we move forward, our commitment to improving FP services must remain unwavering. By focusing on education, access, quality care, supportive policies, and cultural sensitivity, we can ensure every individual has the opportunity to make empowered decisions about their reproductive health. Together, we can work toward a future where FP methods are understood, accessible, and embraced by all enhancing both the health of

Sincerely,



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ACKNOWLEDGEMENT

This Family Planning training module has been revised to improve and strengthen the knowledge, attitudes, and skills of service providers, incorporating recent global updates in FP service provision.

This revision was made possible through the leadership of the Reproductive Health, FP, and Adolescent & Youth Health Desk, with the guidance of the Maternal, Child, and Adolescent Health Lead Executive Office of the Ministry of Health.

The review process involved meaningful engagement with federal and regional health sectors, donors, implementing partners, and professional associations through multiple consultative sessions. The active participation and valuable inputs from experts within MOH-MCAH LEO, donors, implementing partners, and professional associations were instrumental in reviewing this module, including both the Participant's Manual and Facilitator's Guide.

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ACRONYMS AND ABBREVIATIONS

AIDS	Acquired Immunodeficiency Syndrome
ANC	Antenatal Care
ART	Antiretroviral Therapy
AYH	Adolescent Youth Health
BPR	Business Process Reengineering
BTL	Bilateral Tubal Ligation
CARMMA	Campaign of Accelerated Reduction of Maternal Mortality in Africa
CEDAW	Convention on the Elimination of All Forms of Discrimination against Women
CIP	Costed Implementation Plan
CPR	Contraceptive Prevalence Rate
CYP	Couple Year of Protection
DD	Demographic Dividend
DHS	Demographic and Health Survey
DMPA-SC/IM	Depot Medroxy Progesterone Acetate-Subcutaneous/Intramuscular
ECP	Emergency Contraceptive Pills
EPI	Expanded Program on Immunization
EPSA	Ethiopian Pharmaceutical Supply Agency
FGAE	Family Guidance Association of Ethiopia
FGM/C	Female Genital Mutilation/Cutting
MOH	Federal Ministry of Health
FP	Family Planning
FP 2030	Family Planning Commitment by 2030
GBV	Gender-Based Violence
HCT	HIV Counseling and Testing
HDA	Health Development Army
HEP	Health Extension Program
HEW	Health Extension Worker
HIUD	Hormonal Intrauterine Device
HIV	Human Immunodeficiency Virus

HMIS	Health Management Information System
HO	Health Officer
HSDIP	Health Sector Medium-Term Development Investment Plan
HTP	Harmful Traditional Practices
ICPD	International Conference on Population and Development
IEC	Information, Education and Communication
IUCD	Intrauterine Contraceptive Device
IVDM	Informed and Voluntary Decision-Making
LAM	Lactational Amenorrhea Method
LMIS	Logistics Management Information System
MCH	Maternal and Child Health
MDG	Millennium Development Goal
MEC	Medical Eligibility Criteria
NGO	Nongovernmental Organization
PASDEP	Plan for Accelerated and Sustained Development to End Poverty
PHCU	Primary Health Care Unit
PLWH	People Living With HIV
PMTCT	Prevention of Mother-to-Child Transmission (of HIV)
PWD	Persons with Disability
RH	Reproductive Health
ROC	Reproductive Organ Cancer
SBC	Social and Behavior Change
SDG	Sustainable Development Goals
SDM	Standard Days Method
STI	Sexually Transmitted Infection
TFR	Total Fertility Rate
UNFPA	United Nations Population Fund
VCT	Voluntary Counseling and Testing
WHO	World Health Organization

EXECUTIVE SUMMARY

Effective Family Panning (FP) services empower individuals and couples to make informed decisions regarding the timing and spacing of their children. This not only improves their quality of life but also contributes to the overall socio-economic development of the country.

Globally, maternal mortality remains a critical issue, with significant variations across regions. In Ethiopia, maternal mortality rates have improved but continue to pose a major health challenge. FP is a key intervention for reducing maternal deaths by preventing unintended pregnancies, spacing births, and lowering the risk of complications.

The revised Ethiopian FP Guideline underscores the importance of accessible and effective FP services, particularly in light of global and national challenges. The primary goal of this update is to align Ethiopia's FP services with global standards and best practices, while addressing the diverse needs of its population and improving maternal health outcomes. Key components of the update include expanding contraceptive options and strengthening strategies for providing FP services during epidemics and in humanitarian settings.

The inclusion of FP in humanitarian contexts is supported by evidence demonstrating its critical role in addressing the reproductive health needs of displaced populations. With rising global displacement and persistent national challenges, ensuring that FP services remain available and accessible is paramount. Self-Care Practices-Such as Self-Injectable Depot Medroxy Progesterone Acetate-Subcutaneous/ Intramuscular (DMPA-SC) are highlighted as strategic mechanisms for maintaining FP services during and after epidemics. Self-injectable contraceptives have been shown to improve uptake and continuation rates, empowering women to manage their reproductive health more effectively.

The guideline also emphasizes task-sharing by Health Extension Workers (HEWs), particularly the expansion of Long-Acting Reversible Contraception (LARC) services at the community level. These updates are grounded in globally generated evidence, ensuring that FP services in Ethiopia remain inclusive, innovative, and effective.

In summary, this guideline seeks to make FP services more comprehensive, accessible, and responsive to the unique needs of various populations, ultimately contributing to the reduction of maternal mortality and the promotion of reproductive health in Ethiopia.

WHAT IS NEW IN THIS EDITION?

This revised guideline introduces updated strategies and initiatives based on global, regional, and national commitments, including:

- Expanded contraceptive method mix: Description of currently available contraceptive methods in Ethiopia, with recommendations for introducing other evidence-based methods.
- FP services during epidemics and humanitarian crises.
- Shift in terminology from "male involvement" to "male engagement" in FP services.
- Innovative FP service modalities**,** such as self-care (e.g., self-injectable DMPA-SC, hormonal IUD/LNG-IUD [clarify if both terms refer to the same method or if distinctions should be made]).
- Expansion of LARC services at the community level through task-sharing by Level IV HEWs (e.g., Implanon removal, IUD insertion and removal), guided by the Health Extension Program (HEP) roadmap.
- Adolescent-focused counseling approaches, including the SMART-START method for adolescent couples.
- Services tailored for populations with special needs, including people with disabilities, adolescents, and youth.
- Introduction of a new insertion site for Implanon NXT.
- Annexes with new tools, including:
    - Job aids (e.g., approaches for missed IUD and implant cases).
    - Report Requisition Form (RRF).
    - Internal Facility Report and Resupply Form (IFRRF).
    - Internal referral form.

CHAPTER 1: INTRODUCTION

The Government of Ethiopia is committed to the achievement of Sustainable Development Goals (SDGs). The latest estimates by the UN indicate that in 2020, the Maternal Mortality Ratio (MMR) in Ethiopia was 267 deaths per 100,000 live births, which is still very high that needs accelerated strategies to reach the target of 70 per 100,000 live births by 2030.[1] These improvements in outcomes can be attributed to better access to evidence-based high impact interventions. Of the high impact interventions, FP could prevent one third of maternal deaths by allowing women to delay pregnancy, avoid unintended pregnancies and subsequent abortions. In this regard, several policies and strategies have been developed with the goal of strengthening the demand and FP service provision.

This guideline seeks to achieve harmony and alignment to the national strategic documents and initiatives including the FP 2030 commitment and other key flagship efforts. Moreover, it intends to incorporate both global and local evidence on FP priorities including the current WHO FP global handbook for providers, 2022, 2nd edition, Health Sector Medium-Term Development and Investment Plan (MOH -2023/24-2025/26), National Reproductive Health Strategy, 2021-2025 and the National RMNACH-N Self-care guideline.

The current guideline focuses on different contraceptive method use, including postpartum FP, post abortion FP and FP counseling. In addition, it provides an update on newly emerged self-injectable contraceptives and hormonal intrauterine device, addresses FP during epidemics and in humanitarian settings.

The guideline aims to provide guidance to national FP program in the delivery of contraceptive services and serves as a framework that supports national targets by implementing best practices and evidence-based interventions. Potential users of the guideline include all cadres of health care providers, policy makers, donors, health managers, FP program coordinators and implementing organizations, Civic Service

[1] WHO, UNICEF, UNFPA, World Bank Group, and UNDESA/Population Division. Trends in Maternal Mortality 2000 to 2020. Geneva, World Health Organization, 2023

Organizations, instructors and students at health training institutions, researchers, monitoring and evaluation experts, and community educators.

1.1. Background

1.1.1. Global context

The global Sustainable Development Goals (SDG) 3.1 targeted reducing maternal mortality ratio to less than 70 per 100,000 live births by 2030, to achieve a better future for all. The SDG aims to ensure universal access to sexual and reproductive health-care services including FP, which will be measured by the proportion of reproductive age women who have their need for FP satisfied with modern methods of contraception[2].

1.1.2. Regional Conventions

In 2006, African ministers of health and delegates agreed on the Maputo Plan of Action. It illustrates the political will within the continent to provide high quality information and services to women, men and young people, targeted to reduce high-risk pregnancies as well as sexually transmitted infections. Improving youth friendly SRH services, reducing the incidence of unsafe abortion and increasing resources to ensure commodity security were the priority actions in the Maputo Plan of Action[3].

The African Union recently updated the Campaign for the Accelerated Reduction of Maternal Mortality in Africa (CARMMA) with a new phase called CARMMA Plus (2021-2030). This phase aims to strengthen efforts to improve Maternal, New-born, Child, and Adolescent health by leveraging high-level advocacy, particularly through Africa's First Ladies. The updated campaign focuses on ensuring that children on the continent survive, thrive, and achieve their full potential. FP interventions are identified as major components to be strengthened to help in the reduction of maternal and child mortality and morbidity[4].

[2] https://www.who.int/data/gho/data/themes/topics/sdg-target-3-1-maternal-mortality

3. African Union (AU), 2006. Maputo Plan of Action for the Operationalisation of the Continental Policy Framework for Sexual and Reproductive Health and Rights 2007-2010: https://www.ippf.org/sites/default/files/maputo_plan_of_action.pdf[3]

[4] African Union (AU), 2021. Launch of the New Phase of the CARMMA Campaign (CARMMA Plus, 2021-2030) : <https://au.int/en/newsevents/20240218/launch-new-phase-carmma-campaign-carmma-plus-2021-2030>

1.1.3. Ethiopia and the International Conventions

Ethiopia as a member of the international community has endorsed and signed agreements promoting and protecting the rights of women. It also ratified the Convention on the Political Rights of Women (CPRW), the Convention on Elimination of All Forms of Discrimination against Women (CEDAW), the Universal Declaration of Human Rights, the Convention on the Rights of the Child, and has endorsed and engaged with the Sustainable Development Goals of 2015.[5]. Ethiopia has also renewed its FP 2020 commitment and extended to 2030.

1.1.4. National Context

1.1.4.1. Policy Environment in the Constitution of Ethiopia

Recognizing the low status of women, the Government of Ethiopia has established constitutional rights, laws, directives, and strategies to empower women and address gender inequity. The Constitution of Ethiopia, adopted in 1995, assures women of equal rights with men in every sphere (Art 35.1) and emphasizes affirmative action to remedy the past inequalities suffered by women (Art. 35.3). It also affirms the need to enforce the elimination of harmful customs and laws that oppress women (Art. 35.4). Article: 35.1:-"Women shall, in the enjoyment of rights and protections provided for by this Constitution, have equal right with men".

Article 35.4:-"The state shall enforce the right of women to eliminate the influences of harmful customs. Laws, customs and practices that oppress or cause bodily or mental harm to women are prohibited." In line with this, the family law was revised in 2000 to align with the national constitutional rights of women, the revised Penal Code (2005), and with international and regional instruments.

Article 35.9:-"To prevent harm arising from pregnancy and childbirth, and to safeguard their health, women have the right to access FP education, information, and services."

[5] UN Women; preliminary gender profile of Ethiopia, Nov 2014:https://www.usaid.gov/sites/default/files/documents/1860/Preliminary%20Gender%20Profile%20of%20Ethiopia%20Nov%2017%20final.pdf

The National Health Policy

The main objective of the National Health Policy is to "ensure provision of comprehensive and integrated primary health care in a decentralized and equitable fashion" (Government of Ethiopia, 1993, revised in 2024). The major emphasis is on health promotion and prevention, focusing on communicable diseases, nutritional disorders, and environmental health problems without neglecting essential curative activities. The policy states that maternal health and child health deserve due consideration. The National Health Policy emphasizes inter-sectorial collaboration, particularly regarding family health.

The National Population Policy's overall objective is to harmonize the rate of population growth with economic development and thereby improve the welfare of the people (Transitional Government of Ethiopia, 1993, revised in 2024). Most of the targets set in the population policy directly or indirectly relate to FP, including reducing the Total Fertility Rate (TFR) and increasing the contraceptive prevalence rate (CPR).

Health Sector Plan

The Ministry of Health (MOH) has been implementing the Health Sector Transformation Plan II (HSTP II) which prioritized FP as an important service to reduce maternal and child morbidity and mortality. The plan is targeted to increase CPR use from 41% to 50% by 2025. Subsequently the MOH developed Health Sector Medium-Term Development and Investment plan (HSDIP, 2023-2026). It also identified major impact indicators, such as reducing the maternal mortality ratio from 267/100,000 to 199/100,000 live births, increasing contraceptive prevalence rate (CPR) from 41% to 45% and reducing teenage/adolescent pregnancy rate from 13.6% to 9%[6]. Moreover, the costed implementation plan (CIP) of FP aims reducing unmet need for FP from 22% to 17%, by 2030.

The National Reproductive Health Strategy

The National RH Strategy (2020-2025) states that the goal of FP is to reduce unintended pregnancies and enable individuals to achieve their desired family size.

[6] USAID, STAT Compiler: the DHS program: https://www.statcompiler.com/en/

To achieve this overall objective, the strategy sets the following interventions:

- Increase access to a wide range of contraceptive methods.
- Improve the quality of FP services.
- Strengthen community-based distribution of contraceptives.
- Enhance FP counseling and education.
- Integrate FP services with other reproductive health services.
- Strengthening monitoring and evaluation systems to track progress and outcomes.

These interventions aim to ensure that individuals have access to the necessary resources and support to make informed decisions about their reproductive health.

The Adolescent and Youth Health Strategy

The population age structure of Ethiopia shows that adolescents make up more than 33.8% of the total population. Recognizing the growing needs of adolescents, the MOH has been implementing the Adolescent and Youth Health Strategy (AYH) to create access to quality youth friendly services. According to the EDHS 2016, the unmet need for FP among married adolescents (15-19 years) is 20%, higher than in other age groups. To address this, the strategy is implementing a tailored counseling guide called Smart Start Counseling, which provides age-appropriate support and empowers married couples to delay their first pregnancy by using contraception. The strategy has been targeted to increase CPR and reduce the unmet need for modern contraception among adolescents and youth.

Financial commitment

Ethiopia signed on to the 2030 FP Financing Commitment. These strategies aim to ensure a steady supply of contraceptives and improve access to FP services across Ethiopia. The four commitments of this strategy are as follows:

- **Increase FP Financing**: Increase funding for FP services by continuing to earmark funds from the treasury and SDG pool fund.
- **Improve Contraceptive Commodity Security**: Increase the "no stock out" status of at least three modern contraceptive methods from 63% to 90%.

- **Improve Access for Adolescents and Youth**: Reduce teenage pregnancy among adolescent girls from 13% to 7% by 2025 and to 3% by 2030.
- **Ensure Availability of Quality FP Information and Services:** Decrease the unmet need for FP from 22% to 17% by 2030.[7]

In summary, the constitution, health-related policies, strategies and financial commitment in Ethiopia cover all major grounds and offer all necessary provisions, creating an enabling environment for quality FP services. However, existing regional disparities and the socio-political environment, lack of quality FP service, financial and human resource constraint, demand side barriers, misconception, and gaps in supply chain management for FP are main challenges for access to and utilization of FP services.

1.1.4.2 Health Service Delivery System

The public health service delivery in Ethiopia follows three-tier system; Primary Health Care (PHC-consisting comprehensive and basic health post, health center, and primary hospital), general hospital and highest specialized hospital. The cadres of health care providers range from HEWs, who carry out their duties at the community and health-post levels to medical specialists. The health infrastructure expansion in the past couple of decades has enabled the health sector to enhance the coverage of primary health care services to almost 100%. However, the provision of quality health care services at all levels is found to be low.

1.1.4.3. Socioeconomic and Gender Perspectives

Gender inequality in Ethiopia remains a major barrier to the country's development. Women and girls are forced to marry at an early age. The 2019 EMDHS indicated that the median age at first marriage is 17.8, below the legal age of marriage. Over 63% of women have experienced Female Genital Mutilation/Cutting (FGM/C), with the highest prevalence in the Somali region at 99%. Other regions with high prevalence include the SNNPR and Afar, both at 91%, while Tigray has the lowest prevalence at 24.2%.[8] Forty-nine percent (49%) of Ethiopian women with FGC aged 15-49 had their genitalia cut before the age of 5 years, while another 24% have experienced it at age 10 or older. Approximately 33% of ever-married women

[7] **Federal Ministry of Health. (2023).** *Ethiopia's Commitment to Family Planning 2030*

[8] Orchid Project (2023) FGM/C in Ethiopia: Country Profile Update, June 2023. Available at www.fgmcri.org/country/ethiopia/.

report that they have experienced either physical or sexual violence while another 26% of ever married women have experienced some form of intimate partner violence. [9]

In most cultures across Ethiopia, men interfere in women's decision-making power and ownership of assets. Men are more likely than women to own land (50 % men and 45% women), use an account in any financial institution (25% men and 15% women), and own mobile phones (55% men, 27%women). Moreover, 38% of ever-married women aged 15-49 reported that they made the decision to marry, while 61% say that their parents made the decision. These all make violence against women in Ethiopia a concern from human rights, economic and health perspectives and pose challenges in achieving gender equality and women's empowerment.

Women and girls are discriminated against their access in health, including FP. This requires a holistic approach to narrow the gender gap and ensure the freedom of choice to access FP as a human right in Ethiopia.

1.1.4.4. Fertility Pattern of Ethiopia

Ethiopia has seen a gradual decline in its fertility rate, with the TFR decreasing from around 5.26 children per woman in 2010 to approximately 4 children per woman in 2021, with a notable difference in fertility rates between urban and rural areas, with rural areas generally having higher fertility rates. The current national population increase is 2.6%.[10] It will take 27 years for the population to double in size, which will make Ethiopia to be the fifth country with the greatest projected population increase by adding more than 83.4 million people from 2018 to 2050 (World Population Data, 2018). This makes the total population of Ethiopia 139.6 million in 2030 and 190.9 million in 2050. With the current age structure, 45% of the population will be under 15 years of age which will put great pressure on the country's economy. As the population that is more youthful is growing, investment in FP will provide an opportunity for a demographic dividend (DD) in the country. Accordingly, it requires concerted effort to increase the country's CPR and reduce unmet need.

[9] Ethiopian Public Health Institute (EPHI) and ICF. 2019. Ethiopia Mini Demographic and Health Survey 2019: Key Indicators. Rockville, Maryland, USA: EPHI and ICF.

[10] https://statisticstimes.com/demographics/country/ethiopia-demographics.php

1.1.4.5. History of Modern FP in Ethiopia

Modern FP services in Ethiopia started by the Family Guidance Association of Ethiopia (FGAE) in 1966. Until 1982, a husband's signed consent was required by FGAE to provide contraceptives to his wife. As a result, 16% of the women who requested contraceptives were not provided with the service. After the requirement of spousal authorization was removed, utilization of FP services increased by 26% within a few months.

In Ethiopia, the history of contraceptive methods spans several decades. The Combined Oral Contraceptive (COC) pills and condoms were first introduced in 1966, followed by injectable in 1980. The Intrauterine Contraceptive Device (IUCD) became available in 1990, while implants (Norplant) were introduced in 1996. Emergency contraceptives made their debut in 2006, and Postpartum Intrauterine Devices (PPIUD) became accessible in 2012.

Since 1980, the MOH has further expanded its FP services through cyclic country support programs by the United Nations Population Fund (UNFPA) and other stakeholders. Following Ethiopia's adoption of a Population Policy in 1993, local and international institutions collaborated with the government in expanding FP programs and services. The National Office of Population then established to implement and oversee the strategies and actions related to the Population Policy.

In 1996, the MOH released the first Guideline for FP Services in Ethiopia to guide stakeholders and expand quality FP services. In this guideline, the MOH designated new outlets for FP services in addition to the pre-existing facility-based and outreach FP services. Moreover, other policy and strategic documents have emphasized integration and linkage of FP services with other RH services, to enhance FP utilization. The second Guideline for FP Services in Ethiopia was revised in 2011, and third in 2020.

1.1.4.6. Current Status of FP in Ethiopia

Ethiopia has shown remarkable progress in reaching women and men with voluntary FP services. The CPR has increased by 37% (8% in 2000 to 41.1% in 2019); Whereas, TFR has declined from 5.9 in 2000, to 4.6 in 2016. Despite the significant progress in CPR, there is a huge variation among urban and rural communities, different regions and across socio demographic status. For example, urban women are much

more likely than their rural counterparts to use any modern method of contraception (50 percent versus 38 percent); only 3.4 percent use FP in Somali compared to 50 percent in Addis Ababa (2019 mini-DHS). Similarly, the unmet need for FP is gradually declining from 37% in 2000 to 22% in 2016 (EDHS). Ethiopian min-demographic survey also indicates that in the year 2019, injectable and implants comprise 27% and 9% respectively compared to the 21% and 3.4% in 2011, this indicates improvement in method mix.

These achievements in the health sector are attributed to the expansion of public and private facilities, the health workforce and health system at the community level. Implementation of the package of family health, expanded training of health workers on FP and expansions on community demand creation for FP services under the leadership of the ministry of health and the financial and technical support from partners and the donor agencies have also contributed to the improved achievements in the reproductive, maternal and child health services.

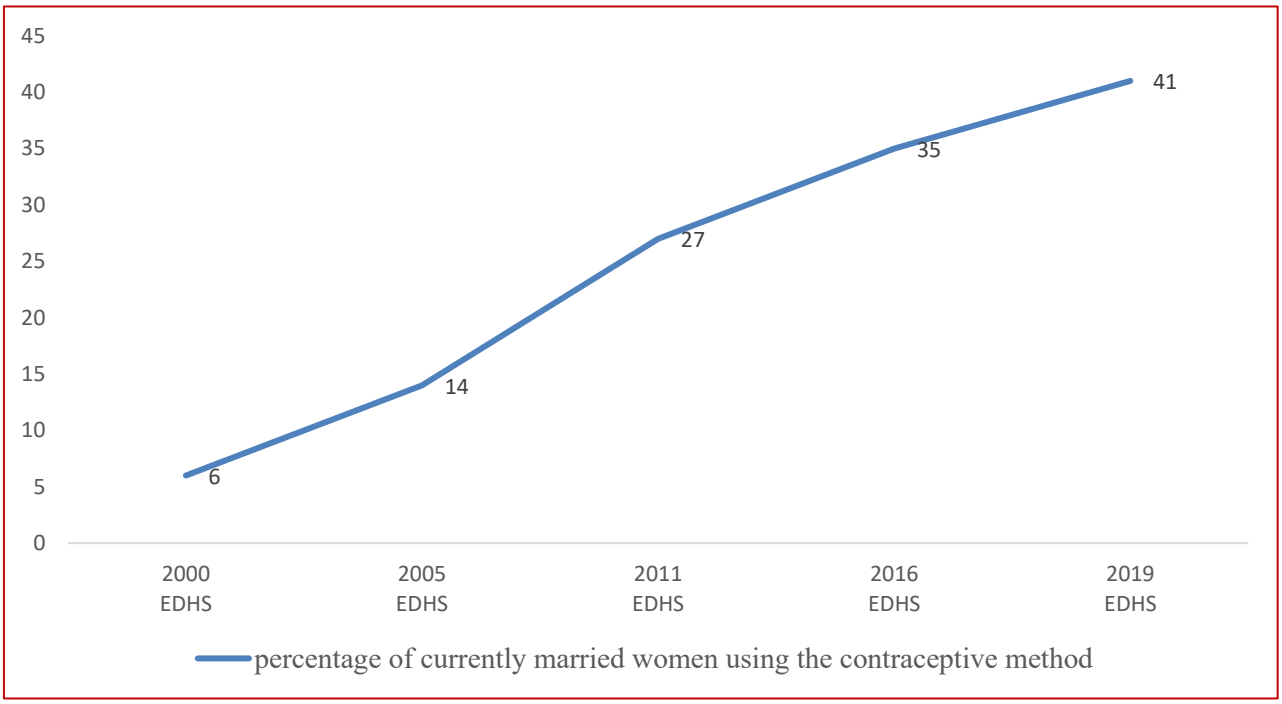


Figure-1: Trend of Modern Contraceptive from 2000-2019 EDHS Data

CHAPTER 2: RATIONALE FOR FPSERVICES

Access to high-quality, affordable sexual and reproductive health services and information, including a full range of FP methods, is fundamental to realizing the rights and well-being of women. Moreover, FP providers are at the core of health system responses to reduce persisting high levels of unmet need for FP. Updating the national FP guideline is essential to ensure that it remains relevant, effective, and responsive to the current needs and challenges.

2.1. Human Rights Rationale

The human rights dimension of FP programs was recognized 50 years ago. The 1968 International Conference on Human Rights brought the Teheran Proclamation, which affirmed FP to be a human right; "…parents have a basic human right to determine freely and responsibly the number and spacing of their children". This right has been reaffirmed and elaborated by various bodies in numerous declarations and conventions over the years, notably the 1979 Convention on the Elimination of all forms of Discrimination Against Women (CEDAW), the 1994 International Conference on Population and Development (ICPD), and the 1995 fourth world conference on Women. Since 1980, Ethiopia has been a signatory to nine of the CEDAW articles[11].

In a similar context, the Constitution of the Democratic Republic of Ethiopia (1995), Article 35.9 states "To prevent harm arising from pregnancy and childbirth and in order to safeguard their health, women have the right to FP education, information, and capacity." Overall, accessing voluntary FP services is recognized to be a human right; and governments are obliged to ensure the accessibility of the service. The FDRE constitution also reaffirms it, and individuals have the right to claim for information and access to quality FP services.

[11] United Nations Treaty Collections: https://treaties.un.org/Pages/ViewDetails.aspx?src=TREATY&mtdsg_no=IV8&chapter

2.2. Women's Health Benefits

Pregnancy and childbirth complications remain leading causes of maternal death globally. Despite a 34% decline in global maternal mortality rates from 2000 to 2020, progress has stalled in recent years, and disparities persist, with sub-Saharan Africa alone accounting for 70% of global maternal deaths in 2020[12]. Evidence suggests that FP interventions contributed to more than a 25% reduction in the maternal mortality ratio. In another context, an estimated 23 million adolescent girls have an unmet need for modern contraception and are at risk of unintended pregnancy. Addressing all these challenges will be possible only through creating access to informed and voluntary FP services[13].

Ensuring access to voluntary FP methods for women and couples has various health benefits to the health of a woman. It helps women preserve their health and contributes to the improvement of the overall quality of their lives. FP helps to space and delay pregnancies for a woman who is at risk of pregnancy related problems. Mothers who have chronic cardiac, renal and respiratory problems, multiparous women who are at risk of post-partum bleeding, and adolescents and youth who want to delay pregnancy will benefit from using a FP method. Unplanned adolescent pregnancies may end in unsafe abortion and its complications. In conclusion, FP saves the lives of women and children, improves the quality of life for all and reduces morbidity and mortality from pregnancy.

2.3. Child Health Benefits

Having too many children places children's health at risk. In some countries, evidence shows that achieving birth spacing of more than two years between pregnancies could reduce child death by up to a third; this is called Healthy Timing and Spacing of Pregnancy. In a similar context, evidence shows that an infant born to a teenage mother is more likely to be born too early and weigh too little at birth and is 24 percent more likely to die in the first month of life than an infant born to a mother aged 25-34 years[14]. Families with fewer, healthier children can devote more resources to providing their children with adequate food, clothing, housing, and educational opportunities. By avoiding too closely spaced

[12] World Health Organization, UNICEF, United Nations Population Fund and The World Bank, *Trends in Maternal Mortality: 2000 to 2020* WHO, Geneva, 2023

[13] Darroch, J.E., V. Woog, A. Bankole, and L.S. Ashford. 2016. Adding It Up: Costs and Benefits of Meeting Contraceptive Needs of Adolescents. New York: Guttmacher Institute. https://www.guttmacher.org/sites/default/files/report_pdf/ adding-it-up-adolescents-report.pdf, accessed 26 June 2017.

[14] Ahmed S, Li Q, Liu L, Tsui A. Maternal Deaths Averted by Contraceptive Use: Results from a Global Analysis of 172 countries.

pregnancies, families will have a greater opportunity to support children, invest in their children's education and bring about better child health. If women had universal access to voluntary FP information and services, infant deaths can be reduced by as much as a fifth[15]. Accordingly, FP service is a very cost-effective investment compared with investments in most other health and social interventions to bring quality of health to a woman and her child.

2.4. Educational Benefits

Adolescents and youth who have the freedom to choose and have access to FP services tend to avoid unintended pregnancies, and educational dropouts. Additionally, having smaller families allows parents to invest more in each child. According to the WHO, children with fewer siblings tend to stay in school longer than those with more siblings. When girls leave school early for marriage and childbearing, it perpetuates the cycle of high fertility, low women's status and high rates of poverty.

2.5. Economic Benefits

The ability to decide when or whether to have children is not only a basic human right; it is also the key to economic empowerment. Investing in expanding access to voluntary FP contributes to better economic outcomes for households, communities, and nations. FP will ensure that women have the freedom to decide when to have children, to be more engaged in various household activities, personal development activities and community participation. On the other hand, investment in FP service saves various expenses that result from social, economic and cultural impacts of unplanned pregnancy and childbirth. According to the world economic forum 2018, for every dollar invested in reproductive health services, $2.20 is saved in pregnancy-related health-care costs.

FP also contributes to the reduction of the national total fertility rate. When developing countries make investments that empower young people to start families only when they are ready, the birth rates decrease, and the proportion of the working-age population increases relative to the dependent population. When there is a simultaneous decline in mortality from communicable and non-communicable diseases, life expectancy in a community increases. This implies that more youth will join the labor force market. If this

[15] World Economic Forum: the economic benefits of family planning, 26, july 2018: https://www.weforum.org/agenda/2018/07/the-economic-benefits-of-family-planning

is matched with an appropriate fiscal policy for better education, quality of health care, innovations in technology and employment opportunities, the growing adolescent and youth population will contribute to the national economy through saving and investment, a concept referred to as the 'Demographic Dividend' by the world bank.[16]

2.6. Environmental Benefit

The global community is working in creating sustainable development which implies responsible utilization of existing resources to meet human development needs while at the same time sustaining the natural systems to provide the natural resources and ecosystem services for the future generation upon which the economy and society depends. Humans are depleting natural resources, degrading soil and water, and creating waste at an alarming rate to meet their needs. This is exacerbated as the fertility rate and the overall global population increase. As FP programs reduce fertility, they can help to relieve the pressures that rapidly growing populations place on environmental resources (land, food and water), reduce strain on community resources (health care, education and agriculture), and lessens pressure on the socio-political system.

[16] *Africa's demographic transition : dividend or disaster (English).* Africa development forum Washington, D.C. : World Bank Group.

CHAPTER 3: OBJECTIVES OF THE FP GUIDELINE

This FP guideline has been revised with the following general and specific objectives.

3.1. General Objective

- To provide a comprehensive framework to ensure the availability, accessibility of high-quality FP services.

3.2. Specific objectives

- Set standards for FP program design and implementation, service provision and monitoring and evaluation of programs.
- Serve as a guide to all cadres of health care providers directly or indirectly involved in the provision of FP services, including for pre-service and in-service training expansion and improve the quality of FP services.
- Guide FP programmers and implementers at government, nongovernment, donors and private sector as well as charity and civic organizations.
- Guide integration of FP services with other RH and non-RH services.

CHAPTER 4: GUIDING PRINCIPLES FOR FP SERVICES

This guideline considers the following FP guiding principles in designing FP programs and expanding access to quality FP services:

- **Availability**: Health care facilities, trained providers and contraceptive methods are available to ensure that individuals can exercise full choice from a range of contraceptive methods, including follow-up and removal services for implants and IUDs.
- **Accessibility**: Health care facilities, trained providers and contraceptive methods are accessible—without physical, economic, socio-cultural or informational barriers.
- **Acceptability:** Health care facilities, trained providers and contraceptive methods are respectful of medical ethics and individual preferences, are sensitive to gender and life cycle requirements and respect confidentiality.
- **Quality**: Individuals have access to full range of quality contraceptive methods which are scientifically and medically appropriate, have access to clear and medically accurate information, and should get the service from technically competent provider at a well-equipped health facility that ensure client-provider interactions.
- **Empowerment**: Individuals are empowered as principal actors and agents to make decisions about their reproductive lives, and can execute these decisions through access to contraceptive information, services and supplies
- **Equity and non-discrimination**: Individuals have the ability to access comprehensive contraceptive services free from discrimination, coercion and violence. FP services should not vary by non-medically indicated characteristics, such as age, geography, language, ethnicity, disability, HIV status, income, and marital or other status.

- **Voluntary informed decision:** Individuals have the ability to access accurate, clear and readily understood information about a variety of contraceptive methods and their use.
- **Privacy and Confidentiality**: In FP service provision, client information should be kept confidential and counseling sessions should be conducted in a private and respectful setting.
- **Referral and Integration with other health services**: There must be a system of intra and inter facility referral for clients with FP needs. Moreover, clients coming for other health services including maternal, child and general health services need to be linked with FP services to enhance access and provide comprehensive care.
- **Voice and participation**: Individual beneficiaries have the ability to participate meaningfully in the design, provision, implementation, and evaluation of contraceptive services, programs and policies.

CHAPTER 5: FAMILY PLANNING SERVICES

5.1. Definition of Family Planning

According to the WHO, FP is defined as the ability of individuals and couples to freely and responsibly decide the number, spacing, and timing of their children. It is achieved through the use of contraceptive methods and the treatment of involuntary infertility.

Key Components:

- **Contraceptive Methods**: A variety of options, including barrier methods, hormonal contraceptiv es, intrauterine devices (IUDs), and permanent methods.
- **Education and Counseling**: Providing comprehensive information on reproductive health, contr aceptive options, and usage.
- **Access to Services**: Ensuring availability and accessibility of FPservices and reproductive health care.
- **Prevention of STIs**: Incorporating strategies for the prevention and treatment of sexually transm itted infections.
- **Infertility Treatment**: Offering support and medical intervention for individuals and couples fac ing infertility issues.
- **Prevention, and screening for reproductive organ cancers:** Focusing on breast and cervical cancer
- **Follow-up and referral:** Ensuring continuous care and appropriate referrals is crucial for effective contraceptive use.
- **Record Keeping and reporting:** This helps in providing personalized care, identifying trends, and improving overall service quality.

Providing these components of FP (FP) ensures a comprehensive approach that supports informed choice, improves health outcomes, and enhances the quality of life for individuals and communities.

5.2. Value Clarification Attitude Transformation (VCAT) on FP

VCAT on FP was designed for a variety of audiences and settings. Facilitators can give one-day stand-alone training for Health care workers at all levels, Health managers, religious leaders, Political leaders, Women groups, HDAs and Community leaders.

VCAT on FP should be integrated with comprehensive FP training package by extending the training duration by one-day to be given for healthcare providers.

5.3. Service Eligibility and Medical Eligibility Criteria for Contraceptive

5.3.1. Service Eligibility

Any person, male or female, who is sexually active married and unmarried, is eligible for FP services, including information, education, and counseling. Users should have the ability to choose and make informed decision.

5.3.2. Medical Eligibility Criteria for Contraceptive

The Medical Eligibility Criteria (MEC) for Contraceptive Use is a set of guidelines developed by WHO to help healthcare providers determine the safety of various contraceptive methods for individuals with specific health conditions or characteristics. These criteria aim to ensure that contraceptive methods are used safely and effectively, while minimizing potential health risks.

Definition

The MEC provides a systematic approach to evaluating the suitability of different contraceptive methods based on a person's medical history and current health status. It categorizes contraceptive methods into four categories:

TABLE 1: CATEGORIES FOR TEMPORARY METHODS

Category	With Clinical Judgment	With Limited Clinical Judgment
1	Use method in any circumstances	Yes (Use the method)
2	Generally, use method	
3	Use of method not usually recommended unless other more appropriate methods are not available or not acceptable	No (Do not use the method)
4	Method not to be used	

TABLE 2: CATEGORIES FOR FEMALE STERILIZATION AND VASECTOMY

Accept (A).	There is no medical reason to deny the method to a person with this condition or in this circumstance
Caution (C)	The method is normally provided in a routine setting, but with extra preparation and precautions.
Delay (D)	Use of the method should be delayed until the condition is evaluated and/or corrected. Alternative, temporary methods of contraception should be provided.
Special (S)	The procedure should be undertaken in a setting with an experienced surgeon and staff, equipment needed to provide general anesthesia, and other backup medical support. The capacity to decide on the most appropriate procedure and anesthesia support also is needed. Alternative, temporary methods of contraception should be provided if referral is required or there is otherwise any delay

How to Use:

- **Assess Health Status**: Evaluate the individual's medical history, current health conditions, and a ny medications they are taking.

- **Refer to MEC Guidelines**: Use the MEC guidelines to determine the appropriate contraceptive method based on the individual's health status.
- **Counsel the Individual**: Provide comprehensive counseling on the chosen contraceptive method , including its benefits, potential side effects, and how to use it correctly.
- **Monitor and FollowUp**: Regularly monitor the individual's health and adjust the contraceptive method if necessary**.**

By following these steps, healthcare providers can ensure that individuals receive the most suitable and safe contraceptive options, promoting better reproductive health outcomes. Specific medical eligibility criteria for each contraceptive method need to follow the World Health Organization (WHO) updated edition Medical Eligibility Criteria for contraceptive use. (Refer to ANNEX-H)

5.3.3. FP Counseling

Definition

Counseling is a type of client-provider interaction that involves two-way communication between a health care provider and a client for the purpose of confirming or facilitating a decision by the client or helping the client address problems or concerns[17].

When providing counseling, health care provider is responsible for helping clients to:

- Assess their own needs for services, information, and emotional support
- Have information appropriate to identify problems and needs
- Make their own voluntary and informed decisions by helping them weigh the options
- Implement their decisions with exploring barriers and helping them develop strategies and skills to overcome those barriers, and carry out their decisions
- Understand all the information they have received and answer questions and addressing concerns.

Counseling Importance: counseling makes the client to be informed and to have voluntary decision making, to have quality FP services and adoption and continuation of FP**.**

[17] *A Global Handbook for Providers (2022 Edition)*. World Health Organization. ISBN: 9780999203705

There are various FP counseling techniques that assist individuals and couples in making informed choices about their reproductive health. Ethiopia has adopted REDI as framework for FP counseling. (See on annex B)

5.4. Provision of Contraceptive Methods

The contraceptive mix in Ethiopia consists of the following commodities and methods:

- Fertility awareness–based methods, such as the standard day's method (SDM), rhythm (calendar) method, two-day method/or cervical mucus method, and sympto-thermal method.
- Lactational amenorrhea method (LAM)
- Male and female condom
- Emergency contraceptives
- Progestin-only pill
- Combined oral contraceptive pills
- Injectable contraceptives (DMPA-IM, SC)
- Implants (Implanon NXT, Jadelle, and Levoplant)
- Intrauterine contraceptive device (IUCDs)
- Levonorgestrel Intrauterine Device (LNG-IUD)
- Bilateral tubal ligation (BTL)
- Vasectomy
- Others as approved by the Ethiopian Food and Drug Authority (EFDA)

Globally there are other contraceptive methods approved and on use, like: - combined patch, combined vaginal ring and progesterone releasing vaginal ring.

5.4.1. Natural FP Methods

Natural FP refers to methods used to prevent or postpone pregnancy by giving attention to natural reproductive events related to fertility. All natural methods except for lactational amenorrhea (LAM) require partners' cooperation. Couple must be committed to abstaining or using another method on fertile days. Couple/client must stay aware of body changes or keep track of days, according to rules of the specific method.

Natural FP methods include Fertility awareness method, Calendar Based, Rhythm, SDM, symptom-based methods, LAM and Withdrawal method.

5.4.2. Short Acting Modern FP

Short acting FP contraceptives used in Ethiopia include:

- Condoms– male / female
- Oral contraceptive pills (Combined and progestin only pills)
- Injectable (-Depot Medroxyprogesterone acetate (DMPA) Intramuscular)
- Injectable (subcutaneous)

5.4.2.1. Barrier Method

Condom

A condom is a barrier method of contraception that is worn over the male sex organ or inserted into the female sex organ during sexual intercourse to prevent pregnancy and reduce the risk of sexually transmitted infections (STIs).

Male and female condoms are both effective methods of contraception). Male condoms, when used correctly, are about 98% effective in preventing pregnancy.

A. Male condom

A male condom is a thin latex sheath that fits over an erect penis. It provides dual protection by preventing pregnancy and reducing the transmission of STIs and HIV. It works by creating a barrier that keeps sperm out of the vagina and prevents infections from spreading between partners.

It is effective method of contraceptive when used correctly. With every act of sex male condom has a typical-use effectiveness of 87% and perfect use of effectiveness 98% in preventing pregnancy.	When used consistently and correctly, condom use prevents 80% to 95% of HIV transmission that would have occurred without condoms.

B. Female condom

Sheaths, or linings, that fit loosely inside a woman's vagina, made of thin, transparent, soft film.

- Has flexible rings at both ends
- One ring at the closed end helps to insert the condom
- The ring at the open end holds part of the condom outside the vagina.

A female condom is a sheath or lining made from a thin, transparent, and soft film, designed to fit loosely inside a woman's vagina. It has flexible rings at both ends to facilitate use. The ring at the closed end aids insertion and helps position the condom properly inside the vagina, while the ring at the open end remains outside, holding part of the condom in place and ensuring it stays secure during intercourse. This design provides a barrier that helps prevent pregnancy and reduces the risk of sexually transmitted infections (STIs). Female condoms are slightly less effective than male condoms, with a perfect-use effectiveness rate of about 95% and a typical-use effectiveness rate of around 79%.

5.4.2.2. Oral Contraceptive Pills (Combined and Progestin Only Pills)

Types of combined Oral Contraceptive Pills

Combined oral contraceptive pills (COCs) and progestin only pills (POPs) are oral contraceptive methods that contain either two or one female sex hormones. The hormones are synthetic estrogens and synthetic progesterone. The Estrogen hormone includes:

- Ethinyl estradiol
- Mestranol

The progestin hormones include:

- Norethindrone
- Norgestimate
- Gestodene
- Desogestrel

In addition to their contraceptive effect, OCPs provide other non-contraceptive health benefits. Hormonal methods do not protect against STIs, including hepatitis B and HIV. Therefore, individuals with risky sexual behavior should use a barrier method (condom) for dual protection against pregnancy and STIs.

Combined Oral contraceptive pills (COCs)

Pills that contain low doses of two hormones-a synthetic progestin and a synthetic estrogen– like the natural hormones' progesterone and estrogen in a woman's body. It works primarily by preventing the release of eggs from the ovaries (ovulation). COCs prevent synthesis of gonadotropins from the pituitary. Hence, ovarian follicles do not mature, and ovulation does not occur.

Effectiveness of COC depends on the user: Risk of pregnancy is greatest when a woman starts a new pill pack 3 or more days late or misses 3 or more pills near the beginning or end of a pill pack. When commonly used, 92 of every 100 women using COCs will not become pregnant. Consistent and correct use (When women remember to take all pills on time)), less than 1 pregnancy per 100 women using COCs over the first year (3 per 1,000 women).

How to Use COCs

A woman can start using COCs any time she wants if reasonably certain she is not pregnant it should be Taken one pill at the same time each day. Eligible targets: Women reproductive age those are eligible according to medical eligibility.

Progestin-only Pills (POPs)

POPs are pills that contain very low doses of a progestin like the natural hormone progesterone in a woman's body. Progestin-only pills (POPs) are also called "minipills" and progestin-only oral contraceptives. It contains 0.025 mg-0.030 mg progesterone of different chemical composition. Nearly all women can use POPs safely and effectively.

POPs effectiveness depends on the user: For women who have monthly bleeding, risk of pregnancy is greatest if pills are not taken every day at the same hour of the day. Breastfeeding women:

- As commonly used, 99 of every 100 women will not become pregnant.
- Women not breastfeeding POPs are less effective: As commonly used, 90 to 97 of every 100 women will not become pregnant.

5.4.2.3. Progestin Only Injectable (DMPA-IM)

Progesterone only contraceptive injections contain progesterone known as depot medroxy progesterone acetate (DMPA). It is highly effective, safe, and Used for three months. It works with the mechanism of Inhibition of Ovulation, Thickens the Cervical Mucus and Thins the Endometrial Lining.

The effectiveness of Depot Medroxyprogesterone Acetate (DMPA) depends on receiving injections consistently and on schedule; the risk of pregnancy increases when an injection is missed. Injectable are highly effective: with typical use, about 97 out of 100 women will avoid pregnancy. When administered on time, fewer than 1 in 100 women using progestin-only injectable will become pregnant in the first year—equivalent to approximately 3 pregnancies per 1,000 women. Women can begin using DMPA at any time, provided it is reasonably certain they are not pregnant and they are medically eligible.

5.5.2.4. DMPA for Subcutaneous Injection

The subcutaneous self-injectable contraceptive is also known as Depot Medroxy progesterone Acetate Subcutaneous (DMPA-SC) that allows individuals to self-administer injections. This method uses a smaller needle to inject the contraceptive just under the skin, rather than into the muscle, which can reduce pain and provide more flexibility in injection sites.

DMPA-SC is available in two injection systems: the Uniject device and prefilled, single-dose conventional syringes. Both use short needles designed for injections just beneath the skin.

- With the Uniject system, the user squeezes a flexible reservoir to push the fluid through the needle. DMPA-SC in this system is marketed under the brand name Sayana Press, which is particularly useful for community-based programs.

Considerations

Training: Initial training is required to ensure correct injection technique.

Side Effects: The most common side effects are mild injection-site reactions. Women may start DMPA-SC at any time, provided it is reasonably certain that they are not pregnant and they are medically eligible.

5.4.2.5. Emergency Contraception

Emergency Contraception is the use of either hormonal pills (estrogen and progestin or progestin alone) or a copper-bearing IUCD to prevent an unintended pregnancy following exposure to unprotected intercourse during emergency conditions.

Types of Emergency Contraceptives

- **Emergency contraceptive pills (ECPs)** are hormonal methods of contraception that can be used to prevent pregnancy following an unprotected sexual intercourse. The risk of unintended pregnancy with the use of combined oral contraceptives for emergency contraception is 2%.

Two types of emergency contraceptive pills:

  - Pills containing a combination of progestin (levonorgestrel or norgestrel) and an estrogen (ethinyl estradiol)
  - Pills containing a progestin only (levonorgestrel or norgestrel)

- **2. A copper-releasing IUD** can also be used within five days of unprotected intercourse as an EC.it is highly effective as ECs. After unprotected sexual intercourse, less than 1% of women are reported to become pregnant if they use a copper- releasing IUD.

  - Emergency contraceptive pills (ECPs) are designed to prevent pregnancy by inhibiting ovulation, fertilization, and/or implantation when taken after unprotected intercourse, including in cases of contraceptive failure (e.g., condom breakage) or sexual assault. They are suitable for use by young adults and can be an effective option if used promptly. However, ECPs are not effective once implantation of a fertilized egg has begun, and they will not cause an abortion. There are no known adverse effects of ECPs on the growth or development of an established pregnancy. It is important to note that ECPs do not provide protection against sexually transmitted infections (STIs) or HIV/AIDS.

  - In addition to ECPs, an intrauterine device (IUD) can also be used as a form of emergency contraception. A copper IUD, when inserted by a healthcare provider within five days of

unprotected intercourse, can be highly effective in preventing pregnancy. It works by altering the environment of the uterus to prevent fertilization and may also inhibit implantation. The IUD is a highly reliable option for emergency contraception and can continue to provide ongoing contraception if left in place. However, like ECPs, it does not protect against STIs or HIV and doesn't cause abortion.

5.4.2.6. Long-Acting FP Methods

Implants

Implants are small plastic rods, each about the size of a matchstick, that release a progestin like the natural hormone progesterone in a woman's body. A specifically trained provider performs a minor surgical procedure to place 1 or 2 rods under the skin on the inside of a woman's upper arm. Implants are one of the most effective and longest-lasting methods. Far less than 1 pregnancy would be expected per 100 women using implants over the first year. They do not contain estrogen and so can be used throughout breastfeeding and by women who cannot use methods containing estrogen.

Types of implants:

- Jadelle: 2 rods containing levonorgestrel (LNG), highly effective for 5 years.
- Implanon NXT (also known as Nexplanon; replaces Implanon): 1 rod containing etonogestrel (ETG), labeled for up to 3 years of use (a recent study shows it may be highly effective for 5 years). Implanon NXT can be seen on X-ray and has an improved insertion device.
- Levoplant: 2 rods containing LNG, labeled for up to 3 years of use.

Addendum procedure should be applied during implant insertion to locate appropriate insertion site.

Women can begin using implants: Without a pelvic examination, without any blood tests or other routine laboratory tests, without cervical cancer screening, without a breast examination and Without a pregnancy test – A woman can have implants inserted at any time, even when she is not having monthly bleeding at the time, if it is reasonably certain she is not pregnant.

5.4.2.7. Copper-Bearing Intrauterine Device

The copper-bearing intrauterine device (IUD) is a small, flexible plastic frame with copper sleeves or wire around it. A specifically trained health care provider inserts it into a woman's uterus through her vagina and cervix. Almost all types of IUDs have one or two strings, or threads, tied to them. The strings hang through the cervix into the vagina. It works by causing a chemical change that damages sperm and egg before they can meet.

In many cases a woman can start the IUD any time if it is reasonably certain she is not pregnant.
CU-IUD is effective: - Less than 1 pregnancy per 100 women using an IUD over the first year (6 per 1,000 women who use the IUD perfectly, and 8 per 1,000 women as commonly used). Even though, the TCu-380A is labeled for up to 10 years of use, Studies have found that the TCu-380A is effective for 12 years.

The IUCD can be used by women of all ages, including adolescents, and is suitable for those who have or have not had children. It rarely leads to pelvic inflammatory disease (PID) or increases the risk of sexually transmitted infections (STIs), including HIV. Additionally, it does not affect fertility, cause miscarriage, birth defects, or cancer, nor does it move to the heart or brain. Both women and their partners typically do not experience discomfort or pain during intercourse. Moreover, it significantly reduces the risk of ectopic pregnancy.

5.4.2.8. Levonorgestrel Intrauterine Device

The levonorgestrel intrauterine device (LNG-IUD) is a T-shaped plastic device that steadily releases a small amount of levonorgestrel each day. A specifically trained health care provider inserts it into a woman's uterus through her vagina and cervix. It is also called the levonorgestrel-releasing intrauterine system, LNG-IUS, or hormonal IUD. It is marketed under such brand names as Mirena, Liletta, Levosert, Kyleena, Skyla, and Jaydess. The Kyleena, Skyla, and Jaydess IUDs and their inserters are slightly smaller

than the Mirena, Liletta, and Levosert. Mirena and Kyleena are approved for up to 5 years of use. Research shows that Mirena remains highly effective for 7 years[18].

LNG-IUD is effective: Less than 1 pregnancy per 100 women using an LNG-IUD over the first year (2 per 1,000 women). Over 5 years of use of the LNG-IUD (Merina), less than 1 pregnancy per 100 women (5 to 8 per 1,000 women). **Women can Start LNG-IUD at any time when it is reasonably certain that she is not pregnant and medically eligible.**

5.4.3. Permanent FP Methods

Permanent method is intended to provide life-long, permanent, and very effective protection against pregnancy. Reversal is usually not possible. There are male and female permanent methods.

5.4.3.1. Female Permanent FP Methods

Female Sterilization: - Permanent surgical contraception for women who will not want more children. It works because the Fallopian tubes are blocked or cut. Eggs released from the ovaries cannot move down the tubes, and so they do not meet sperm. One of the most effective contraceptive methods but carries a small risk of failure (Less than 1 pregnancy per 100 women over the first year after having the sterilization procedure (5 per 1,000).

There are 3 surgical approaches most often used:

- **Mini laparotomy**: involves making a small incision in the abdomen. The fallopian tubes are brought to the incision to be cut or blocked.
- **Laparoscopy:** involves inserting a long, thin tube containing lenses into the abdomen through a small incision. This laparoscope enables the doctor to reach and block or cut the fallopian tubes in the abdomen
- **Postpartum Tubal ligation:** When tubal ligation is performed during a cesarean Chapter (C-Chapter) or laparotomy.

[18] FDA label update reflects Bayer's commitment to providing options for long-acting contraception. Business Wire. August 18, 2022. Accessed August 22, 2022.

Female sterilization is safe for all women who have received proper counseling and provided informed consent. It's important to ensure that the woman fully understands the decision to avoid any future regrets. The procedure can be performed at any time, provided she is not pregnant and has no medical conditions that would contraindicate it.

Female sterilization is a safe procedure that does not weaken women or cause lasting pain in the back, uterus, or abdomen. It does not involve the removal of the uterus or lead to hormonal imbalances, and it does not affect menstrual cycles, weight, appetite, or appearance. Additionally, it has no impact on sexual behavior or drive and significantly reduces the risk of ectopic pregnancy. **All clients who are eligible and want to receive Female Sterilization must have signed an informed consent (attached at the Annex E).**

5.4.3.2. Male sterilization (vasectomy)

Vasectomy (male sterilization) is a permanent contraception for men who will not want more children. The procedure is through a puncture or small incision in the scrotum, the provider locates each of the 2 tubes that carry sperm to the penis (vas deferens) and cuts or blocks them by cutting and tying them closed or by applying heat or electricity (cautery).

Vasectomy is the most effective methods but carries a small risk of failure: -Among the partners of men who have vasectomies, far less than 1 in every 100 will become pregnant in the first year of use of the method. Vasectomy is not fully effective for 3 months after the procedure and couples need to use backup method for 3 months. It is safe for All Men with proper counseling and informed consent; any man can have a vasectomy safely. **All clients who are eligible and want to receive Male sterilization must have signed an informed consent (attached at the Annex E).**

A vasectomy does not involve removing the testicles; instead, it blocks the tubes that carry sperm. It does not affect sex drive, sexual function, or performance, and does not lead to weight gain, weakness, or loss of masculinity. The procedure does not cause any long-term diseases and does not protect against STIs, including HIV.

5.5. Postpartum and post abortion FP

Postpartum and postabortal FPare essential components of maternal and child health. These services aim to provide contraception to women who have recently given birth or undergone an abortion to prevent unintended pregnancies and support healthy timing and spacing of pregnancies**.**

Both approaches are vital in reducing maternal and neonatal morbidity and mortality, empowering wom en to make informed reproductive choices, and improving overall family health and well-being.

5.5.1. Postpartum FP (PPFP)

Postpartum Contraception: is the initiation and use of FP methods in the first year after delivery to prevent unintended pregnancy when another pregnancy can be harmful to the mother or a breastfeeding baby.

Postpartum Method Choice (see at annex H, WHO MEC,)

- All non-hormonal methods can safely be used by breastfeeding women. Since they do not interfere with a woman's ability to breastfeed, the quality/quantity of breast milk, or produce any adverse effect on infant growth and development.

- Non-hormonal methods include: lactational amenorrhea method (LAM), condoms (male or female), postpartum intrauterine contraceptive devices (PPIUD), voluntary surgical contraception (VSC-male or female) and PPIUD with Dedicated IUD insertion tube (Pregna).

5.5.2. Post Abortion FP Method

Post abortion contraception is the initiation and use of FP methods, most often immediately after treatment for abortion within 48 hours, or before fertility returns (2 weeks post abortion). Woman has important choices and opportunities, before, during, and after abortion care. However, if there is a need to wait; women could have any FP method after abortion care if she met the eligibility criteria.

5.6. Counseling and Linkage to other SRH Services

Counseling and linkage to other SRH services including sexual dysfunction is important. Health facilities mainly focus on providing access to reproductive health service. The focus towards creating access to sexual health including counseling for sexual dysfunctions, gender-based violence, and the prevention of

sexually transmitted infections is usually overlooked. This guideline recommends that health facilities integrate sexual and reproductive health into FP services. Health care providers also need to identify and manage sexual problems and/or ensure referral where needed.

5.7. Prevention and Management of Infertility

World Health Organization (WHO) has considered Infertility a Public Health problem. It has been defined as "a disease of the reproductive system translated into the inability to obtain a pregnancy after 12 months or more of regular sexual intercourse and without using contraception" and reported that the prevalence of infertility is 17.5% of people experienced infertility between 1999 and 2021.[20] Based on the 2016 Ethiopian Demographic and Health Survey (DHS), the prevalence of infertility among women who had been married for at least five years and had never used contraceptives was 7.6%.

The role of FP in the prevention of infertility is through the promotion of responsible sexual behavior, use of condoms (dual protection), STI counseling, screening and treatment when indicated. Health professionals should make clear that contraceptives do not cause infertility. Despite the presence of variations related to age and health status of the woman, most modern contraceptive methods do not cause a significant delay in the return to fertility. However, if a client presents with infertility, provide appropriate counseling and information on where to get services.

5.8. Prevention and Screening for Reproductive Organ Cancers

Cervical cancer is a significant public health issue in Ethiopia. According to recent reports, cervical cancer is the second most common and deadly cancer among women in Ethiopia. The prevalence of cervical cancer is increasing due to factors such as population growth, aging, and the presence of well-established risk factors.[21]

FP offers a unique opportunity to screen for cervical cancer and teach the client to do self-examination for breast cancer. Where facilities exist, women should be encouraged to have Pap smear or visual inspection

[19] World Health Organization 2025, 'Infertility,' WHO, <https://www.who.int/news-room/fact-sheets/detail/infertility>.
[20] World Health Organization 2025, 'Infertility,' WHO, <https://www.who.int/news-room/fact-sheets/detail/infertility>
[21] Awoke Derbie, et al. Cervical cancer in Ethiopia: a review of the literature. Cancer Causes & Control. 2022;34(1):1-11.

of the cervix using acetoacetic acid (VIA) or Lugols Iodine Solution (VILI) based on the recommended schedule. Health workers should educate women and their families about reproductive organ cancers (ROCs) and the benefits of screening. Women found to have ROC should receive their treatment either in the same facility or be referred as urgently as possible.

5.9, Follow-up and Referral

Ensuring follow up, referral and continuity of care is an important component of FP services. Ensuring follow-up, referral, and continuity of care is a critical component of FP services, as it helps to address the diverse and ongoing needs of individuals seeking reproductive health support. FP is not just about providing information or services at a single point in time, but rather about creating a comprehensive, long-term approach to reproductive health. This includes continuous access to a variety of contraceptive methods, regular follow-up to monitor individual health and satisfaction, and referrals to specialized care when needed. (See at Annex-F&G, internal and intra-facility referral forms)

Referrals are crucial in connecting individuals with additional services, such as counseling, STI testing, maternal health care when required. Furthermore, continuity of care fosters a trusting relationship between healthcare providers and individuals, promoting consistent care across different health systems and facilitating informed decision-making about FP.

CHAPTER 6: FAMILY PLANNING FOR CLIENTS WITH SPECIAL NEEDS

FP service providers have a duty to ensure equitable access to services for all, including groups with special needs. These guidelines focus on the following categories of clients considered to have special needs:

6.1. Adolescent and Youth

Age groups of 10-24 years are called young people, with 10-19 years considered adolescents (for whom we use the term contraceptive rather than FP) and 15-24 years defined as youth.

In Ethiopia, modern contraceptive use among adolescents and youth has increased over the last two decades, with higher prevalence among sexually active unmarried youth compared to married youth. According to the 2019 mini-EDHS, only 36.5% of married young women aged 15-19 use modern contraceptives, influenced by religious and cultural pressures to prove fertility. Teenage pregnancy and motherhood rose from 12% in 2011 to 13% in 2016. The unmet need for contraception remains high at 23.4% for urban youth. Injectable are the most common method for both married and unmarried youth, followed by male condoms and implants. The adolescent age group of 15-19 years has higher unmet needs than the youth age group of 20-24 years. This indicates that adolescents face more barriers like, lack of knowledge and information, cultural and religious beliefs, concerns over privacy and confidentiality, economic barriers, and judgmental attitudes from healthcare providers and community members to accessing contraceptives, highlighting the need for improved service provision[22].

Appropriate sexual and reproductive health services, including contraception, should be available and accessible to all adolescents. To make services friendly to adolescent and youth, you can:

- Show young people that you enjoy working with them.
- Offer services that are free or as low cost as possible.
- Offer a wide range of contraceptive methods, including long-acting reversible methods.
- Counsel in private areas where you and the client cannot be seen or overheard.
- Ensure confidentiality.
- Reassure the client that the services are confidential.

[22] Federal Ministry of Health. (2021). *National Adolescents and Youth Health Strategy (2021-2025)*. Addis Ababa: Federal Ministry of Health.

- Listen carefully and ask open-ended questions such as "How can I help you?" and "What questions do you have?"
- Use simple language and avoid medical terms.
- Use terms that suit young people. Avoid such terms as "FP," which may seem irrelevant to those who are not married. Preferred terms such as contraception are better.
- Welcome partners and include them in counseling, if the client desires.
- Try to make sure that a young woman's choices are her own and are not pressured by her partner or her family.
- Take time to fully address questions, fears, and misinformation about sex, sexually transmitted infections (STIs), contraceptives, and implement a comprehensive health approach that includes accessible, inclusive, and informed reproductive health services for young people.

Important considerations in provision of contraceptive to young people:

All Contraceptives Are Safe for Young People

- Young people can safely use any contraceptive method. Age is not a medical reason for denying any method to adolescents.
- Young women are often less tolerant of side effects than older women. With counseling, however, they will know what to expect and may be less likely to stop using their methods.
- Unmarried young people may have more sex partners than older people and so may face a greater risk of STIs. It is important when counseling young people to consider STI risk and how to reduce it.

6.2. HIV Positive and High Risk for HIV Clients

6.2.1. HIV Positive Clients

For HIV positive clients, dual method use helps to prevent transmission of HIV to an uninfected partner. The fertility intentions of HIV positive clients are varied and the right of all women to decide their number and timing of children, regardless of HIV status, needs to be respected. Avoiding unintended pregnancy in HIV-positive women using FP is one of the four prongs of PMTCT. Regardless of their use of ART, HIV positive clients can start and continue to use most contraceptive methods safely. Health care providers

working in ART clinics should inform and educate HIV positive clients about the prevention of unintended pregnancy and the use of FP. Use of hormonal contraceptives in all HIV-positive women is recommended regardless of ART use as the benefits of using contraceptives outweigh the potential risk of unintended pregnancy. However, assessing eligibility using MEC is recommended for each FP method.

6.2.2. High Risk for HIV

Factors that may indicate a higher risk for HIV include having multiple sexual partners, engaging in unprotected sex with a partner who is living with HIV or with a partner whose HIV status is unknown, those currently infected with a sexually transmitted infection (STI), individuals who engage in unprotected sex in exchange for money or goods, those who use injecting drugs and sharing needles for injection and survivors of sexual violence.

All forms of contraception can be safely used by clients at high risk for HIV in the absence of any other medical or physiological contraindications. Condoms (both male and female) are the only currently available dual protection method for preventing both STIs (including HIV) and unintended pregnancy. Additionally, counseling on the benefits of dual method use – condoms plus another form of contraception-helps clients make informed choices regarding the prevention of HIV, other STIs, and pregnancy.

6.3. Survivors of Sexual Violence

Sexual violence is a public health problem and a violation of human rights. Sexual violence is associated with numerous physical, psychological, and emotional consequences. Unintended pregnancy is one of the consequences of sexual violence. Hence, emergency contraception should be availed for all victims of rape who are at risk of pregnancy. Emergency contraceptive pills (ECPs) and the IUCD are the available types of emergency contraception. Whenever pre-packaged ECPs are not available, substitution with combined oral contraceptive or progesterone only pills (COC & POP) are possible. There are no known medical conditions where ECP use is contraindicated. Providers should refer the national management

guideline for victims of sexual assault for provision of linkage to comprehensive sexual and reproductive services for survivors.[23]

6.4. Infibulation

Female genital mutilation/cutting (FGM/C) is a practice internationally recognized as a human rights violation, and it involves altering the female genitalia for non-medical reasons. Infibulation is the type four FGM/C and the most severe form of female genital cutting, is about 6.5% among women aged 15-49 in Ethiopia.[24] FP service providers might have difficulty in providing IUDs in women with infibulation. Therefore, such clients need counseling on other choices of contraceptive methods.

6.5. Persons with Disability

Women and men with disabilities can and want to be productive members of society. Health service providers must ensure that people with disabilities have access to counseling on sexual and reproductive health services and access to informed and voluntary FP services. According to the Central Statistics Agency (CSA) and other national surveys, approximately 17.6% of the population in Ethiopia lives with disabilities, and this includes women of reproductive age.[25]

The following are lists of recommendations to address the needs of people with disability:

- Special consideration should be given to individuals who are mentally challenged or those with psychiatric disorders who might require specialized counseling. People with intellectual disabilities have the same rights as other people to make their own decisions about contraception, including sterilization. They may need special support to do so. For a client with an intellectual disability who is unable to communicate her or his preferences clearly, someone whom the client trusts should participate and help to make an informed choice that is as consistent as possible with the client's preference, especially for the choice of sterilization.

[23] Federal Ministry of Health (FMOH), Ethiopia. Comprehensive Sexual and Reproductive Services for Survivors of Gender-Based Violence. Addis Ababa, Ethiopia: FMOH; 2016.

[24] Ethiopian Public Health Institute (EPHI), ICF. Ethiopia Mini Demographic and Health Survey 2019: Key Indicators Report. Rockville, Maryland, USA: EPHI and ICF; 2019

[25] Central Statistics Agency (CSA) 2020, Representation of Persons with Disabilities in Ethiopian Labor Force: A Review of National Surveys on Disability Statistics, Addis Ababa: Central Statistics Agency

- Where the nature of the disability does not allow for informed choice (e.g., severe mental challenge), a FP method should be provided only after full discussion with all parties, including parents, or next of kin, legal appointed representatives or guardians, depending on the degree of the mental disability. In the absence of these caretakers, the provider may decide on a method choice in the best interests of the client with serious mental disability.
- Some drugs used to treat mental disorders may affect the bioavailability and efficacy of hormonal contraceptives, in which case it is important to consider alternatives.
- As much as possible, FP methods that do not seriously demand user compliance (e.g. Injectable, IUD, and implants) should be encouraged to ensure efficacy and compliance. However, for individuals with severe limb and pelvic disabilities, the provision of implants and IUDs may be a challenge. In such cases, providers should consider alternative contraceptive methods.
- The health care facilities need to be more accessible and friendly to PWDs such as providing wheelchair ramps, adjustable examination couches, and/or staff who are trained in sign language. If possible, print materials should have simple graphics, large print, and Braille, and information should be available in audio formats.
- Consider the reproductive rights of the individual in all decisions.

6.6. Homeless

In an expanding rural urban migration and fast urbanization, more people earn their life through the informal sector and are becoming homeless. Homeless individuals face unique challenges that hinder their access to FP services. These challenges include unstable living conditions, inconsistent access to healthcare, and increased vulnerability to various health risks. Therefore, tailored approaches (address their high-risk behavior and increase access of FP through outreach services, mobile clinic etc.) are essential to address their specific needs and ensure they receive comprehensive support and care.

6.7. Women near Menopause

Menopause usually occurs between the ages of 45 and 55. About half of women reach menopause by age 50. By age 55, some 96% of women have reached menopause. To prevent pregnancy until she is no longer

fertile, an older woman can use any method if she has no medical condition that limits its use. By itself, age does not restrict a woman from using any contraceptive method.[26]

Special Considerations about Method Choice (ANNEX.I WHO MEC)

Women aged 35 and older who smoke-regardless of how much-should not use COCs, Women aged 35 and older who smoke 15 or more cigarettes a day should not use monthly injectable. And Women aged 35 or older should not use COCs, monthly injectable, the patch, or the combined vaginal ring if they have migraine headaches. Lack of regular cycles before menopause makes fertility awareness method more difficult to use.

When Can a Woman Stop Using FP?

Because bleeding does not come every month in the time before menopause, it is difficult for a woman whose bleeding seems to have stopped to know when to stop using contraception. Thus, it is recommended to continue using a FP method until 12 months with no bleeding have passed.

Hormonal methods affect bleeding, and so it may be difficult to know if a woman using them has reached menopause. She can switch to a non-hormonal method. She no longer needs contraception once she has had no bleeding for 12 months in a row. Copper-bearing IUDs can be left in place until after menopause. The IUD should be removed 12 months after a woman's last monthly bleeding.

[26] *A Global Handbook for Providers*. 2022 edition. World Health Organization. **Section: Family Planning for Menopause.**

CHAPTER 7: FAMILY PLANNING DURING EPIDEMICS AND IN HUMANITARIAN SETTINGS

7.1. Providing FP Services during an Epidemic

Epidemics of life-threatening infectious diseases are becoming more common, and cause significant disruptions to the health care system, affecting the delivery of both routine and urgent health services and care. FP providers need to balance the demands of responding to outbreaks, while simultaneously maintaining FP services in all three phases of an epidemic response: mitigation and preparedness, emergency, and post-emergency.

When healthcare systems are overwhelmed by an outbreak, both direct deaths from the disease and indirect deaths from other preventable or treatable conditions are likely to increase. Family planning (FP) supports both physical and mental health by reducing maternal and infant morbidity and mortality through the prevention of unintended pregnancies and unsafe abortions. During an epidemic, unintended pregnancies carry an increased risk. FP is a vital, life-saving service and must be maintained throughout an epidemic. Maintaining FP services during such times is crucial to prevent unintended pregnancies and safeguard reproductive health. Here are some key considerations:

- **Continuity of Services:** FP services should be maintained throughout all phases of an epidemic, including mitigation and preparedness, emergency, and post-emergency periods.
- **Self-Administered Contraceptives:** Expanding access to self-administered contraceptives-such as pills, condoms, and injectable like DMPA-SC-can help maintain contraceptive use when healthcare access is limited.
- **Digital Health Technologies:** Utilizing digital health technologies can improve access to FP services and support remote consultations. Examples include digital tools for contraceptive delivery in humanitarian settings.
- **Multi-Month Supplies:** Providing multi-month supplies of contraceptives reduces the need for frequent visits to healthcare facilities.

- **Infection Prevention:** Ensuring that safety protocols are followed protects both healthcare providers and clients from infection.
- **Community Engagement:** Engaging community health workers and volunteers supports FP efforts by providing education and counseling.

7.2. FP in Humanitarian Setting

The Ethiopia Humanitarian Response Plan (HRP) 2024 outlines the efforts to address the multiple challenges faced by the country, including climate change impacts, conflicts, and security issues. The HRP 2024 includes a comprehensive focus on sexual and reproductive health (SRH), gender-based violence (GBV), and sexual exploitation and abuse (SEA) needs. The plan recognizes the significant GBV and SRH needs arising from multiple shocks, including internally displaced persons (IDPs), returning IDPs, returning migrants, and host communities[27].

Violence against girls and women in humanitarian setting increases the risk of unintended pregnancy and its consequences. Demand for contraception found to be high during emergencies. Evidence suggests that nearly 40% of women experiencing displacement across diverse settings want to avoid becoming pregnant in the next two years.[28] This makes contraception an essential part of any emergency health response.

The Minimum Initial Services Package (MISP) for sexual and reproductive health (SRH) is a set of life saving SRH services and priority activities to be implemented during the onset of every humanitarian emergency. The MISP has six objectives classified under three categories; the sixth objective of MISP is preventing unintended pregnancies.[29]

At the onset of an emergency, it is essential to ensure the availability of contraceptives to prevent unintended pregnancies. The SRH Coordinator, health program managers, and service providers should implement the following interventions:

[27] **United Nations Office for the Coordination of Humanitarian Affairs (UNOCHA). (2024).** *Ethiopia Humanitarian Response Plan 2024*.
[28] Family Planning 2030 (FP2030). Global Progress and Commitments: Emergency Preparedness and Response. Washington, D.C.: FP2030; 2024.
[29] Ministry of Health (MOH), Ethiopia. (2023). Emergency Preparedness and Response Guide for Sexual and Reproductive Health in Emergency Situations. Addis Ababa: MOH.

- **Availability of Methods:** Ensure a range of long-acting reversible and short-acting contraceptive methods including male and female condoms and emergency contraception—are available at primary healthcare facilities to meet demand.
- **Information Materials:** Ensure that relevant information, education, and communication (IEC) materials are available and accessible.
- **Contraceptive Counseling:** Provide counseling that emphasizes informed choice and consent, effectiveness, client privacy and confidentiality, equity, and non-discrimination.
- **Community Awareness:** Ensure the community is informed about the availability of contraceptives for women, adolescents, and men.

CHAPTER 8: INTEGRATION OF FAMILY PLANNING INTO OTHER HEALTH SERVICES

FP service integration- It refers to "an incorporation of managerial or operational changes to health system/service to bring together inputs, delivery, management and organization of FP service with the aims of improving access, quality and efficiency thereby maximizing use of resources and opportunities"[30].

The national RH strategic plan (2021-2025) identifies integrated service delivery as essential for ensuring that every woman, newborn, adolescent, and youth receive high-quality care. Consequently, it is imperative to continually seek opportunities to connect and incorporate reproductive health services with both health and non-health interventions.

There remain missed opportunities for FP services at all levels of health facilities, leading to increased unintended pregnancies, maternal, neonatal, and under-five deaths that could have been prevented. Therefore, increasing FP service utilization through integration and linkage strategies is crucial to enhance access to essential health care services and reduce these high mortality rates. The key benefits of integration of FP and other Health services:

- Improve quality of care and clinical outcomes
- Broadens client opportunity for FP information and services,
- Addresses missed opportunities and increase uptake of contraception
- Improve client satisfaction
- Ensure efficient use of resources
- Increase CPR and decrease the unmet need for FP

Integration and linkage of FP service should be considered in a health facility that has any one or more of the following services:

[30] *Family Planning Service Integration National Implementation Guideline*. Maternal, Newborn, Child and Nutrition Directorate, Ministry of Health. Addis Ababa: Federal Ministry of Health.

8.1. Child health, Immunization, and other Reproductive Health Services

Child health and immunization services create a good opportunity for the provision of FP information and counseling. Furthermore, programs that address harmful traditional practice (HTPs,) gender-based violence (GBV), prevention and management of infertility, screening for Reproductive organ Cancer (ROC), life skill education and other RH services create opportunities for FP services. Hence, these services utilized to address issues related to FP.

8.2 Antenatal Care, Delivery Care, and Postpartum Care

Evidence suggests that there is high-unmet need for post-partum FP in Ethiopia. Despite a high proportion of women exposed to health providers when utilizing maternal health services, only a small proportion receive FP information during these interactions. Accordingly, this guideline advises integrating postpartum FP (PPFP) into maternal, newborn, and child health services to increase the likelihood that every new mother will leave the clinic having made an informed choice about FP.

During antenatal care visits, providers should discuss the benefits of spacing between births, counsel on family size, post-partum FP and exclusive breastfeeding along with having a skilled birth attendant. Similarly, during delivery and post-partum period, providers should support breast-feeding; introduce lactational amenorrhea method (LAM) and other immediate postpartum contraceptive options; injectable, implants, post-partum IUCD and others as per the updated MEC criteria 2015.

8.3. Comprehensive Abortion Care

A woman seeks safe abortion or post abortion care largely because of unintended pregnancy. Abortion and post abortion care may be the first encounter of a woman within the health system, so providers should utilize this opportunity to counsel and provide FP services to the woman or couple. The Technical and Procedural Guidelines for Safe Abortion Services in Ethiopia recommends that a woman provided with the choice of contraception immediately after abortion (MOH, 2023). Global evidence indicates that post abortion women accept FP methods at a higher rate when the method is offered at the same time and location as abortion or post abortion care treatment prior to discharge from the facility. Therefore, this guidance recommends that post abortion FP counseling and services be provided to women seeking abortion or post abortion care services prior to being discharged from the treating facility.

8.4 .HIV Testing and Counseling (HTC)

HTC services can be good entry points for FP services, and vice versa. Both HIV and unintended pregnancy are, in most cases, the consequences of unprotected sex. Integrating HTC and FP service delivery is cost-effective and enables maximum utilization of health care in one visit. Health care workers who provide services for people living with HIV should have basic knowledge and counseling skills to provide FP services. Facilities should also create the enabling environment to strengthen the integration of FP services. Women at high risk of acquiring HIV infection can generally use all methods of contraceptives. With minimum input, both types of providers can deliver services to clients seeking HTC and FP services in one stop.

8.5. Integration with Other Health Services (Inpatient and Outpatients)

It is true that most health facility users visit a facility for an outpatient clinical service. According to 2024 Annual performance report of MOH, Ethiopia; it is reported that, more than 186 million clients visited outpatient departments out of which, 52% were women visit. With a minimum input to health care providers in an outpatient department, integration of FP services will provide an opportunity to expand access to quality FP service provision. Integration of services at OPD might be either using an internal referral mechanism (especially for long-acting contraceptives) or direct provision of FP services at OPD level (especially for short acting FP services) depending on the context.

8.6. School Health Program

Recognizing the more than 28 million adolescent and youth population that attend school, the ministry of health has started implementing a school health program in collaboration with the ministry of education. This school health program (SHP) aims to guide service providers and administrators at different levels of school to provide quality, standardized promote, preventive, and curative health services to school students at secondary and tertiary levels of education in a healthy environment. Sexual and reproductive health interventions including FP services are components of the basic service packages in secondary and Tertiary School. The health sector and partners working in the SRH area need to support capacity building, service provision and referral service to ensure access to quality SRH services was including FP.

CHAPTER 9: FAMILY PLANNING SERVICE DELIVERY MODALITIES

All public health institutions in Ethiopia-rural and urban, hospitals, health centers, health posts; school clinics, workplace-based clinics, youth centers clinics, private clinics and clinics owned by non-governmental organization shall provide FP services. FP services shall deliver through the following service delivery modalities:

9.1. FP Services by Level of Health Facility

The provision of FP services is dependent upon the integration of services throughout the health care system, starting from the community level to specialized referral hospitals. In addition to outpatient clients, FP counseling and services should be made available to postpartum women, post abortion women, and individuals with special needs. All health workers providing FP services should have competency in clinical and counseling skills. Table 3 and 4 is a summary of the types of recommended services to be rendered and the types of providers who should be staffing the different levels of care. The skill level and task analysis summarized by provider in Table 5.[31]

Table 3: Organization of Services by Level of Care in the Public Health Structure

Level of facility	Type of health personnel available for FP	FP services
Basic Health Post	Family Health Professional Nurse Level 4 HEW	• Awareness and demand creation of FP. • Adolescent and Youth friendly counseling and care. • Conduct general physical and pelvic examination • Comprehensive FP service, including insertion IUCD and implants. • Referral based on client's complaint for STI to HCs for further treatment • Refer to health center for permanent methods. • Do planning based on local data

[31] Ethiopian Standard Agency, 2021

Level of facility	Type of health personnel available for FP	FP services
		• Keep clients record and share monthly reports to nearby health center
Comprehensive Health Post	Family Health Professional Midwife, Comprehensive Nurse, Health Officer, Level 4 HEW	• Awareness and demand creation of FP • Adolescent and Youth friendly counseling and care • Comprehensive FP service including insertion and removal of implants and IUCD • Conduct general physical and pelvic examinations, including VIA/VILI • Referral based on client's complaint for STI to HCs for further treatment • Provide syndromic management of STIs, HIV counseling and treatment • Refer to health center for permanent methods • Conduct planning based on local data • Keep clients record and share monthly reports to nearby health center
Health Center	Health Officers (HOs), Midwives, Clinical Nurses, Public Health Nurses General Medical Practitioners (GMPs)	The above activities, plus: • Manage complications and side effects • Conduct screening for ROCs • Train community level workers and junior health professional • Provide on the job training • Conduct clinical mentorship and supportive supervision to health posts
Health Centers with CEMONC	Health Officers (HOs), Midwives, Clinical Nurses, Public Health Nurses	The above activities, plus: • Provide permanent methods

Level of facility	Type of health personnel available for FP	FP services
	GMPs, Integrated Emergency Surgical Officers (IESO)	
Primary Hospital	IESOs, GPs, HOs, Midwives, Clinical Nurses, Public Health Nurses, Family medicine specialist	The above activities, plus: • Conduct work-ups for infertility • Provide permanent methods of contraception • Manage complicated STIs • Receive referrals • Manage complications and side effects • Train health workers on comprehensive contraception • Establish training centers • Conduct screening for reproductive organ cancers • Conduct clinical mentoring to catchment health centers
General and Referral hospital	FP subspecialists, Obstetrician-Gynecologists, GMPs, HOs, Midwives, Clinical Nurses, Public Health Nurses	The above activities, plus: • Manage complications and side effects of contraceptive methods. (including:-Deep Implant and missed IUD) • Manage reproductive organ cancers • Clinical mentorship to catchment-based district hospitals • Pre-service training on comprehensive contraception • Management of infertility • Perform research
Pharmacy	Pharmacist	Provide: • OCP • Emergency contraceptive • SC-DMPA*

* Only refilling is to be done by the pharmacists, not the initiation.

TABLE 4: SERVICE ORGANIZATION OF PRIVATE FACILITIES, NGOS, HIGHER LEARNING INSTITUTIONS AND WORK BASED FACILITIES BY LEVEL OF CARE

Level of facility	Type of health personnel	Available (minimum) FP services
Medium clinic	GP HO Midwife Clinical Nurse	• Counsel on FP and RH • Counsel on use of fertility awareness methods • Distribute male and female condoms, oral contraceptives, (including ECPs) • Provide injectable, including self-injectable and educate on how to inject SC-DMPA • Provide insertion and removal of Implants • Provide insertion and removal of IUCD • Manage side effects of contraceptive • Record keeping and monthly reports to nearby health center • Referral service for permanent contraceptive and other SRH services
SRH Medium Clinic	GP HO Midwife Clinical Nurse	The above, plus: • Vasectomy
Specialized Clinic (MCH)	Ob-gyn specialist Midwife Nurse IESO GP HO Anesthetist	The above, plus: • All methods of FP including fertility awareness methods • Generating evidence and lessons learned • Manage complications and side effects of contraceptive methods

Level of facility	Type of health personnel	Available (minimum) FP services
		• Manage reproductive organ cancers (ROCs) • Establish training centers
General, Specialized Hospital, MCH Specialty Centre (MCH)	Variable types and numbers of professionals (including specialists and subspecialists)	• The above, plus: • Have training centers and train health professionals • Manage complications and side effects of contraceptive methods (including deep implant removal) • Manage reproductive organ cancers (ROCs) • Manage complications and side effects of contraceptive methods • Manage reproductive organ cancers (ROCs) • Perform research
Pharmacy	Pharmacist Pharmacy technician	Provide: • OCP • Emergency contraceptive • SC-DMPA*

* Only refilling is to be done by the pharmacists, not the initiation.

TABLE 5: TASK ANALYSIS FOR PROVISION OF FP SERVICES

Task	Task Provider category						
	Obstetrics and gynecology specialist	GMP /ISO	BSc(HO, Nurse, Midwife)	Diploma (Midwife, Nurse)	HEWL4	HEWL3	Pharmacist
Client assessment							

History taking	✔	✔	✔	✔	✔	✔	✔
Physical examination	✔	✔	✔	✔	✔	✔	X
Bimanual pelvic exam	✔	✔	✔	✔	✔	✔	X
MEC assessment	✔	✔	✔	✔	✔	X	X
Counseling	✔	✔	✔	✔	✔	✔	✔
Provision of FP services, by method:							
Natural	✔	✔	✔	✔	✔	✔	X
Condoms	✔	✔	✔	✔	✔	✔	✔
Pills	✔	✔	✔	✔	✔	✔	✔
Emergency contraceptives	✔	✔	✔	✔	✔	✔	✔
Injectable (IM-DMPA and SC-DMPA)	✔	✔	✔	✔	✔	✔	✔#
Implant-NXT	✔	✔	✔	✔	✔	X	X
Other implants insertion	✔	✔	✔	✔	✔	X	X
Implants removal	✔	✔	✔	✔	✔	X	X

IUCD insertion and removal	✓	✓	✓	✓	✓	X	X
BTL	✓	✓	✓	X	X	X	X
Vasectomy	✓	✓	✓	✓	X	X	X
Other RH services							
Syndromic management of STIs	✓	✓	✓	✓	X	X	X
Management of complicated STIs	✓	✓	✓	X	X	X	X
Cancer screening	✓	✓	✓	✓	X	X	X
Treatment of ROCs	✓	X	X	X	X	X	X
Management of infertility	✓	X	X	X	X	X	X
Pain medications							
Nonnarcotic analgesics	✓	✓	✓	✓	✓	✓	X
Narcotic analgesics	✓	✓	✓	X	X	X	X
Local anesthesia	✓	✓	✓	✓	✓*	✓*	X

Management of complications and side effects	✓	✓	✓	✓	✓^	✓^	X
Follow-up care	✓	✓	✓	✔	✔	X	X
Universal precautions	✓	✓	✓	✓	✓	✓	✓
Integration of FP and other RH services	✓	✓	✓	✓	✓	✓	NA
Instrument processing	✓	✓	✓	✓	✓	✓	NA
SBCC	✓	✓	✓	✓	✓	✓	NA
Recording and reporting	✓	✓	✓	✓	✓	✓	✓
Training junior health professionals and community health worker	✓	✓	✓	✓	✓	✓	✓

Key; ✔ = Roles expected to be performed by the category ✔*= Roles expected to be performed by the category after additional in-service training ✔^ = Reassurance and analgesics for mild side effects and refer X = Roles not expected of the category ✔#=only refilling of SC-DMPA is to be done by the pharmacists, not the initiation.

9.2. Outreach and Mobile Services

Outreach health program is when a health facility team arranges for service provision at a catchment area. Mobile FP service delivery is a service provided by a team of competent providers who travel from a center to a community that has limited or no FP services. The Ethiopian government has been working on integrating FP services into primary health care, and mobile services are part of this effort to increase

access and utilization. This includes providing FP services through mobile health platforms and outreach programs to reach remote and underserved areas (pastoralist, homeless and others).

9.3. Social Marketing

Social marketing is a strategy used to promote, distribute, and sell contraceptives at an affordable price through existing commercial channels. Social marketing used for the promotion and sales of condoms, pills, ECPs and injectable through pharmacies, drug stores, and rural drug vendors to complement facility-based services.

9.4. Social Franchising

This type of partnership focuses on building the capacity of existing private-sector health facilities and their staff providers to deliver selected health care services that require a clinical procedure. Organizations experienced in providing FP services can collaborate with the private sector and strengthen their system in expanding access to quality informed and voluntary FP services.

9.5. Workplace-based FP Services

Factories, enterprises, large farms, and industrial parks employ large numbers of young people, predominantly women, with limited access to SRH/FP services. Hence, ministries and agencies (e.g., Trade and Industry, Agriculture, Energy and Mines, Transport, and Communications) should ensure the availability of SRH services. Availing FP services at the workplace have the benefit of accessing an easy-to-reach, known population of workers. It potentially saves employees' time, minimizes lost productivity, and has the benefit of reaching more potential users. Facilities at workplaces must register and function based on the staffing and facility standards of the MOH. The Ministry of Health at each level should closely work with stakeholders to support workplace-based FP service.

9.6. School-Based Health Services

The provision of sexual and reproductive health services in school settings and in higher learning institutions, have the benefit of accessing an easy-to-reach and known population of youth. Student clinics in academic institutions not only provide young people with objective information on sexuality and responsible sexual behaviors, but they also can offer opportunities for offering HIV testing, STI prevention

and early management, and FP services based on the education sector's guidance. This will help for further reduction of unintended pregnancy, avoid unsafe abortion, and help reduce school dropout. School based interventions should include building the capacity of health providers working in student clinics and equipping the school-based clinics with necessary materials, equipment and supplies. In addition, it should involve establishing and /or strengthening referral arrangements between student clinics and health facilities (Hospitals and health centers) for FP services that do not offer in the student clinics. Prevention of unintended pregnancy should be a main component of the intervention in schools. The intervention should use various strategies tailored towards the need and status of in-school youth as per the national school health strategies. A comprehensive life skills education approach should be the main strategy for demand creation and behavioral change in the area of SRH.

9.7. Private Sector

Both the public and private sectors have critical roles to play, but the private sector, and in particular the for-profit private sector, is often forgotten when FP programs are discussed. The Ethiopian private sector contributes approximately 16% to FP services.[32] It is important to make sure that private providers have the training opportunities similar to those that are in the public sector. They should also contribute to the data generation by reporting required data to the government.

9.8. NGO Health Facilities

Under the strategic leadership played by the government of Ethiopia, NGOs have been the major contributors to Ethiopia's FP success. NGOs shall continue to take part in FP programs through capacity building focusing on quality counseling and service provision, quality assurance, strengthening commodity supply and service provision in alignment with national policies and programs

9.9. Digital Platforms

In Ethiopia, women can purchase FP(FP) methods digitally through various platforms. One official way is by using the Smart Start tool, which provides counseling and options for contraceptive methods. The focus group for accessing Smart Start services primarily includes rural Ethiopian girls aged 15-19 and

[32] Federal Ministry of Health (FMOH), Ethiopia. (2016). Sources of Family Planning: Ethiopia. Addis Ababa: FMOH

their Husbands. There are also a number of other digital platforms available widely, which includes free mobile phone applications as well as hotlines that can be accessed for information, counseling and linkage to services.

CHAPTER 10: QUALITY OF CARE IN FAMILY PLANNING

The HSDIP (Health Sector Development and Investment Plan) continues to emphasize quality as a central pillar of its transformation agendas. To facilitate the implementation of quality interventions across all levels, the Ministry has introduced the Ethiopian National Health Care Quality and Safety Strategy 2021-2025. This strategy redefines quality as "comprehensive care that is measurably safe, effective, patient-centered, uniformly delivered in a timely manner, affordable to the Ethiopian population, and efficiently utilizes resources and services." It provides a roadmap for addressing key quality challenges and accelerating nationwide health care quality improvements. Quality is often viewed as the best strategy for creating demand. The quality improvement process aims to continuously enhance practices until they the service provision reached the desired standard. Quality services meet the needs of clients and adhere to accepted standards and guidelines. Standards for FP services and programs are a set of the minimum acceptable levels of performance and expectations for providing quality FP services and implementing effective and efficient programs in Ethiopia.

The following lists of FP quality standard are set on the bases of right based approach of FP stated on the national guideline.[33]

[33] Ministry of Health (MOH), Ethiopia. (2023). *National Family Planning Quality Standards Implementation Guideline*. Addis Ababa: MOH.

10.1. Essential Resources and Infrastructure

Health facilities should have well-maintained infrastructure with essential services, including adequate ventilation, reliable power, and proper hygiene facilities. FP rooms, including procedure rooms, should be located near other Maternal, Newborn, And Child Health (MNCH) areas, ensuring they are clean, well-ventilated, well-lit, and designed to provide privacy and effective care. Basic amenities such as water, electricity, and sanitation should be safe, functional, and consistently available.

10.2. Voluntary Informed Choice

Voluntary informed choice emphasizes clients selecting the best contraceptive method for their personal reproductive health needs, based on a thorough understanding of their options without coercion. This focus improves FP outcomes by involving clients in decisions about their care, increasing satisfaction, use, and continuation of contraceptive methods. Informed choice also boosts clients' confidence and commitment to their healthcare decisions. The absence of quotas, targets, and incentives in FP programs ensures voluntary decision-making rights

10.3. Contraceptives, Supplies, Equipment, and Instruments

Ensuring the availability of a full range of contraceptive methods, supplies, medicines, equipment, and instruments in sufficient quantities improves access to diverse contraceptive options. This allows clients to have their needs and changing preferences met throughout their reproductive lives. An effective logistics system is also crucial, as it provides accurate forecasting, strengthens the capacity of facility staff to report on contraceptive commodities, and ensures timely and accurate ordering. This system helps achieve commodity security in health facilities. Adequate stocks and a functional logistics management system are essential for providing high-quality FP services and meeting client satisfaction.

Ethiopia's strategy on method mix involves the introduction and promotion of contraceptive methods mix including new contraceptive, which were not available before such as hormonal intrauterine devices and SC-DMPA to broaden contraceptive options.

10.4. Respectful Care

Respectful care is a crucial component of maintaining reproductive health rights, ensuring protection against abuse, discrimination, and other gender-based violence, thereby respecting the integrity of all FP clients. Providers who follow medical ethics and culturally appropriate practices during counseling and procedures are preferred by clients. Additionally, maintaining client confidentiality and privacy during treatment and counseling is essential for all clients, including marginalized and high-risk individuals. Ensuring privacy and confidentiality allows clients to accept or decline counseling, examination, or treatment, and ensures that no client is subjected to mistreatment based on personal attributes. Healthcare providers must respect client privacy and confidentiality, ensuring personal information is not shared without full and informed consent.

10.5. Competent and Motivated Health Care Provider

Availability of healthcare staff around the clock is essential for optimal FP services and timely management of side effects and complications. Qualification, training, experience and Continuing Professional Development (CPD) will improve staff performance. A staffing policy specifying the number, types, and competence of staff ensures an adequate skill mix.

Both clinical and non-clinical staff should be oriented to their roles, with updated staff rosters displayed. Staff should have job descriptions listing necessary competencies and qualifications. Programs for recruitment, retention, professional development, and continuing education are essential, along with regular orientation on protocols, mentorship, supportive supervision, annual performance appraisals, and recognition of good performance. This ensures clients always have access to competent providers for FP services and management of complications.

10.6. Client Safety

Clients should receive medications that meet their clinical needs and individual requirements for an adequate period, at the lowest cost to them and their community. Ensuring client safety is vital for quality FP services and involves continuous improvement from learning errors and adverse events. Client safety reduces costs related to harm and increases healthcare efficiency, restoring community trust in health systems. FP providers should properly screen clients for medical eligibility and follow infection

prevention and waste disposal standards. Facilities must have all appropriate commodities stored correctly and competent providers to manage and report complications and adverse events. This comprehensive approach ensures the provision of safe, effective FP services.

10.7. Accessibility (Time, Affordability & Geography)

Geographic accessibility of health facilities is vital for obtaining services, measured by population coverage, travel time, and distance to facilities. Timely service provision (24/7) reduces delays and improves service uptake, client satisfaction, and reduces method discontinuation. Financial protection through insurance and eliminating user fees increases access to and utilization of FP services. The opening hours should be convenient for all clients, with clear signage and trained providers available 24/7. Facilities must be within two hours' walking distance, provide outreach services, and be accessible to people with disabilities. Financial affordability ensures services are accessible to low-income groups, adolescents, and other vulnerable populations, with clear guidance and insurance programs in place.

10.8. Services Integration

This approach aims to address other sexual and reproductive health (SRH) needs of FP clients and vice versa, maximizing service opportunities. Women's health services are often segmented, so this standard can be applied in two ways: a one-stop-shopping approach or integration with other SRH services. All FP clients should receive requested services either from a single provider with the necessary capabilities or through integrated SRH services. This ensures comprehensive care, reduces missed opportunities, and enhances client satisfaction. Health facilities should establish referral systems, train providers on integrated services, and ensure functional internal referrals to meet clients' needs effectively.

10.9. Quality Data for Decision Making

Measuring performance against plans is crucial for tracking progress and ensuring accountability in Family planningprograms. Monitoring and evaluation allow for impact assessment and necessary corrective actions. Properly maintaining clients' medical records and FP service records, conducting regular data quality assessments, and performing performance monitoring and quality audits are essential to achieving these goals. Ensuring thorough and accurate record-keeping, data quality, and continuous performance evaluation will enhance the effectiveness and accountability of FP programs.

CHAPTER 11: SOCIAL AND BEHAVIOURAL CHANGE (SBC) FOR FAMILY PLANNING

Social and Behavioral Change (SBC) is a discipline that applies a deep understanding of human and societal behavior, along with evidence-based interventions, to promote the adoption of healthy behaviors and influence the social norms that underpin them.

In many countries, entrenched social norms, religious beliefs, and cultural traditions affect attitudes and behaviors, preventing people from seeking, accessing, and using essential family planning and reproductive health (FP/RH) information and services. SBC combines strategies, approaches, and methods that enable individuals, families, groups, organizations, and communities to actively participate in achieving, protecting, and sustaining their own health.

SBC interventions strengthen:

- Demand for FP
- Client-provider communication
- Couples' communication
- Engagement of community leaders and other influencers of behaviors and norms.

In the 2020-2025 strategy, the Ministry of Health has utilized the socio-ecological model to identify existing gaps and propose recommendations for a social and behavioral communication strategy. As a result, individual, community, organizational, and socio-economic level strategies have been recommended to enhance awareness and demand for FP services.

11.1. Individual Level Interventions

At the individual level, accurate knowledge about fertility and FP is essential to informed choice. Other individual factors influencing someone's ability to reach their fertility intentions include beliefs, attitudes, and personal agency, including self-efficacy. This explains the link between these individual level factors and FP outcomes.

11.2. Interpersonal Interventions

Male engagement in FP

Men and boys' engagement is crucial both from the programmatic point of view and as a process for bringing about gender equality in men and women's reproductive rights and responsibilities. It helps not only in accepting a contraceptive method, but also in its effective use and continuation. Accordingly, men should be addressed in FP programs and services as users, promoters, and supporters. Therefore, the following should be considered to ensure male engagement in FP:

- Identify and strengthen community structures to engage men in reproductive health specifically on FP.
- Focus on a gender equity approach to expand FP services and ensure male engagement
- Provide space in all levels of health facilities for couples' counseling in FP.
- Encourage couples' communication regarding fertility and FP, so that decisions reflect the needs and desires of both men and women.
- Provide men with information that enables them to responsibly participate in FP use and decision-making.
- Make information on FP, STIs/HIV, and other RH issues available to men, encourage men to accompany their partners on FP visits, encourage and help to develop men as responsible adults and parents, so they can play an important role in preventing unintended pregnancy and STIs.
- Involve men in the design and implementation of FP and RH services and allow them to express the ways in which they can be encouraged to take more responsibility.

11.3. Community Level Interventions

The community should be aware of the overall benefits and availability of FP services. FP programs and services, including SBC activities, should respect the customs and traditions of the community. Community involvement is a key to dispelling rumors and misconceptions, and thereby developing ownership of FP programs by the community for successful and sustainable outcome.

11.4. Organization Level Interventions

The health sector is one of the major sources of health and health related information for the community. In this regard, enhancing the capacity of the health care providers on interpersonal communications and counseling skills is very important. The major approaches for expanding facility-based access to health information include:

- Standardize waiting areas at each level of the health care delivery system (PHCU, Hospitals and the like)
- Strengthen the production and distribution of standard audio video communication materials to be used in the waiting areas.
- Support health service providers with appropriate and standardized communication materials, job aids and print materials.
- Enhance client provider counseling beyond the health sector; men, women and girls, adolescent and youth are found in different sectors across the community.

11.5. Socio-cultural, Economic Communication Channels

For effective SBC, a mixed indoor and outdoor communication channels approach should be used. SBC messages should be standardized, audience specific (age, gender, educational level, marital status) and culturally sensitive and acceptable. The message should be clear and easily understandable. The target groups should include policy makers, health care providers, opinion leaders, religious bodies, women and girls, men, adolescents and youth, communities, media personnel, and partner organizations.

CHAPTER 12: FAMILY PLANNING PHARMACEUTICALS LOGISTIC MANAGEMENT SYSTEM

To ensure sustainable FP service provision, every healthcare facility must have an adequate and uninterrupted supply of essential medicines, medical supplies, equipment, and commodities that meet standards of safety, quality, and efficacy.

12.1. Integrated Pharmaceuticals Logistics System (IPLS)

The IPLS is a single pharmaceutical reporting and distribution system operated based on the overall mandate and scope of the Ethiopian Pharmaceutical Supplies Services (EPSS). Ethiopia is currently using an integrated pharmaceuticals logistics system (IPLS) for managing contraceptives together with other program commodities. Contraceptive security exists when people are able to choose, obtain and use contraceptive products whenever they need them. A strong supply chain requires routine and accurate updates of records, timely reporting, and generating information for decision-making. In the IPLS decision to resupply commodities, bin card and stock card must be completed. Data from these records are used in monitoring stock levels and ordering refill quantities. The basic components of IPLS which includes: - Logistics Management Information System (LMIS), Inventory Control System and Storage of Pharmaceuticals in the IPLS. Contraceptives should be always in full supply to maintain appropriate level of stock at all-time using the necessary inventory control tools.

12.2. Logistics Management Information System (LMIS)

Logistics Management Information System (LMIS) is the collection, organization, and reporting information to other levels in the system to make decisions that govern the logistics system and ensure that all six rights (rights of quantity, condition, place, time, cost and goods) are fulfilled for each client. Logistics information should be collected and reported monthly by health posts, every other month by health centers and hospitals using logistics management information system (LMIS) forms.

LMIS is to support the management of essential pharmaceuticals. Three essential data items are required to run a logistics system and, therefore, must be captured by the LMIS. These three essential data items are:

- Stock on Hand: Quantities of usable stock available at a particular point in time.
- Consumption Data: The quantity of pharmaceuticals used during the reporting period.

- Losses/Adjustments: Losses are products removed from stock for reasons like expiry, theft, or damage, recorded as negative numbers. While Adjustments are products received from non-EPSS sources or issued to others, including corrections for errors.

The ministry of health fully integrates FP commodities into the LMIS and service delivery point tracing tools at all levels of supply chain system. Each activity in the logistics cycle, therefore, contributes to excellent customer service and to ensuring commodity security. This will be done through proper product selection, quantification, procurement, storage and distributions of FP commodities.

- **FP Commodities Selection:** health programs must select FP commodities. In a health logistics system, a national formulary and therapeutics committee, pharmaceutical board, board of physicians, or other government-appointed group may be responsible for product selection. Most countries have developed FP commodity lists patterned on the World Health Organization (WHO) Model List. Products selected for use will impact the logistics system, so the logistics requirements must be considered during the product selection.
- **FP Commodities Quantifications:** determining the quantity of FP commodities needs is the key for resource mobilization and the national pooled procurement. Through proper documentations of service and logistics data, there will be used for quantifications of the products for upcoming years. Service logistics and demographic data should be used for the national FP commodity quantification.
- **FP Commodities Procurement**: commodities will be procured as per the national commodity procurement system through EPSS and partners. The resources required for procurement will be mobilized from government treasury (including federal and regional government), partners, donors, and co-financing scheme.
- **FP Commodities Storage:** FP commodities should be stored in accordance with IPLS and store appropriately. The storage condition of commodities will follow the national standard storage guideline designed by EFDA. The key activities of storage are **receiving the FP commodities** from the respective EPSS hubs and **inventory control** with effective inventory management and prevent shortages and reduce losses, using various techniques like regular inventory counts and

proper reporting formats like bin card, RRF and IFRRF for consistent availability. (see at annex I and J).

- **FP Commodities Distributions:** Distributions of FP commodities should be conducted as per integrated pharmaceutical and logistics supply (IPLS) guidance. The FP commodities distributed from central EPSS to hubs and facilities will be managed by EPSS.

According to the IPLS, EPSS will deliver pharmaceuticals directly to health facilities whenever the facilities are geographically accessible. For facilities that are inaccessible, the pharmaceuticals will be delivered to the Woreda Health office (for 'woreda pass-through' health facilities) or to their catchment health centers or hospitals, from which the inaccessible facilities will collect them.

TABLE 6: SUMMARIZES THE MINIMUM AND MAXIMUM STOCK AND EMERGENCY ORDER POINTS AT DIFFERENT HEALTH FACILITIES:

Level	Review Period	Maximum Months of Stock	Minimum Months of Stock	Emergency Order Point
Health Centers, Hospitals, private ART clinics	Every other month	4 months	2 months	0.5 months (= 2 weeks)
Health Posts	Monthly	2 months	1 months	0.25 months (= 1 week)
Non ART private clinics	Monthly	2 months	1 months	0.25 months (= 1 week)

The following are the major forms used for LMIS at various levels:

- **Internal Facility Report and Resupply Form** (IFRR): This form is completed by different units within a health facility; exemplified by various dispensary units report to a central store within a health center or hospital. (see at annex-I).
- **Health Post Monthly Report and Re-supply Form (HPMRR):** This form is used for resupplying pharmaceuticals, including contraceptives from health posts to health centers.

Report and Requisition Form (RRF): This form is completed by the Health Centers and Hospitals and sent to the regional PSA for order processing every other month for direct delivery facilities. For management and supervision purposes, a copy of the health center report and request and a copy of each

health post report are sent to the Woreda Health Office. A copy of the hospital report and order is sent to the RHB/ZHD. (See at annex J).

TABLE 7: REPORTING FREQUENCY AT DIFFERENT LEVEL OF HEALTH FACILITIES

Unit	Reports to/Orders from	Every	Until the	Using this report form
Health Post	Health Center	Month	5th day after the reporting period	Health Post Monthly Report and Re-supply Form
Health Center (Direct)	Woreda Health Office / EPSA	2 months	10th day after the reporting period	Report & Requisition Form
Health Center (Nondirect)	Woreda Health Office	2 months	5th day after the reporting period	Report & Requisition Form
Woreda Health Office (for nondirect delivery Health Centers)	EPSA	2 months	10th day after the reporting period	Individual Health Center Report & Requisition Form
Hospital (public and private)	Regional Health Bureau / EPSA	2 months	10th day after the reporting period	Report & Requisition Form
Private ART clinics	EPSA/Regional Health Bureau	2 months	10th day after the reporting period	Private Clinic Report and Requisition Form (PCRRF)
Private non-ART clinics	Health Centers/Woreda Health Office	1 months	5th day after the reporting period	Private Clinics Report and Resupply Form (PCRRSF)

CHAPTER 13: MONITORING AND EVALUATION OF FAMILY PLANNING SERVICES

Monitoring and Evaluation is one of the key components that helps policy makers, program managers, and health workers to track performance, to know areas of improvement and help for an overall strategic decision making at all levels. As part of the overall health sector transformation plan, routine health management information system (HMIS), and population and facility-based surveys (EDHS, PMA) were some of the information sources for measuring FP program. Monitoring and evaluation of FP interventions helps to know the FP performance, tells which activities are more efficient and effective to meet the program objectives, informs to make data driven decisions, and identify challenges, opportunities and strengths during program implementation. In this regard, this guideline recommends the use of interventions like review meeting, clinical mentoring and supportive supervisions, in addition to the major indicators captured through the routine health information system and surveys for an immediate program support.

The successful implementation of the national FP strategy relies on robust monitoring and evaluation system carried out as an integral part of the strategy. Continuous monitoring of progress and evaluations of outcome and impacts will provide the required evidence for decisions that foster effective, efficient and synergistic implementation of programs. Moreover, it will be integrated into knowledge management efforts that will help document lessons and sharing of experiences both nationally and at the international area

13.1. Supportive Supervision

The MOH, RHBs, zonal health departments, Woreda health offices and primary health care units will conduct regular, quarterly, and annually supportive supervision and monitoring visits to all FP providing facilities. During these visits, supervisors will meet with providers to observe and collect data, address implementation challenges and offer feedback. Supportive supervision will also be used to strengthen capacity building efforts of FP service provisions. Additionally, the MOH will convene stakeholders and partner annually to address progress, share lessons learned and adjust challenges encountered during implementations.

In addition to integrated supervision, there should be program specific supportive supervision for the targeted interventions like IUCD scale up and self-administered DMPA. These will allow program specific support at all levels for smooth implementation of the initiatives. A comprehensive supportive supervision checklist should be prepared by engaging all stakeholders at national and regional level. Post training follow-up should be conducted at all levels to monitor and evaluate the training program effectiveness and quality of the services after completing the training.

13.2. Mentorship

The ministry of health has launched a national catchment based clinical mentorship guideline for maternal and reproductive health services, mainly for integrated RMNCAH services. This is a team based technical support and program review from a nearby health facility which is identified to have a better performance on FP services, coverage as well as quality. This guideline recommends that clinical mentors and supervisors use the data they have for the improvement of FP service provision.

13.3. Review Meeting

Programs are reviewed at different levels of intervention to check how they are being implemented and to identify successes and major challenges. Review meetings with health care workers, partners, and health managers are held at district, zonal, regional and national levels and conducted at various frequencies each year. The MOH holds a biannual review meeting with implementers, partners and donor agencies where FP service performance is one of the priorities reviewed.

The meeting needs to be supported by up-to-date data and reports collected from national, regional and health facility level related with comprehensive FP services. The MOH, regional, RHBs, EPSS, EFDA, health facilities and partners should be parts of the performance review meetings that can be organized at national and regional levels. This guideline recommends that FP program review should be continued at national and subnational level as one of the priority programs reviewed.

13.4. Health Management Information System (HMIS)

Ethiopia has been implementing the health management information system since 2008 to capture and provide core indicators to improve the provision of health services. The system is a major source of information for monitoring and adjusting policy implementation and resource use. Recently, the health

information system, which uses HMIS, has been replaced by the district health information system (DHIS2), which provides aggregates of health information at a facility level. The FP service provision is one of the services which need to be captured through the DHIS system. In this guideline, the major data that need to be captured in the DHIS and the tools used to collect client's information are described to help health care workers and health managers to follow FP services are well recorded and used for decision making. The major records are categorized as individual client's records, aggregate registers, and tally sheets.

13.4.1-Community Health Information System (CHIS) and Electronics Community Health Information System (eCHIS)

The Ministry of Health has developed a community health information system (CHIS) alongside the Health Extension Program (HEP). This system is utilized to monitor the program's progress and is integrated into the Health Management Information System (HMIS). Various strategies were employed to implement CHIS in different community settings. An approach called the Family Folder was introduced in agrarian areas to document HEP services provided by HEWs (HEW) since 2012. Despite initial success, challenges were identified after a decade of CHIS implementation. To address these issues, an electronic version of CHIS (eCHIS) was introduced as part of the information revolution in 2018.

13.4.2. Health Information System at Facility Level

Table 8: Shows individual's FP recording tools at health facilities:

Integrated individual Folder	Capture basic demographic info, which includes a summary sheet for each visit, filled out by service providers immediately after service.
Women's Card	This card records client's socio demographic and health history, including screening for FP, past/current FP methods, physical exam findings, and current FP method. The follow-up Chapter records history and exam findings during the visit.
Appointment Card	A small card used to remind clients of their next appointment. The card includes the client's demographic info, appointment date, and reason for the appointment.
Referral form	Used to transfer basic information from referring health facilities to accepting health facilities. It is based on the community-based health information system (CHIS) from HP to HC.
Family Health Card (at health post level)	Issued to any client visiting a health post, helping the HEW capture demographic info, FP provision, and long-term FP removal. The card is kept in a tickler box with appointments or placed in the back of the family folder.

13.4.3. Registers

A. FP Register

FP Register is a longitudinal register that is used to capture HMIS data related to FP services. The information required to complete the FP register is obtained from woman's card. The register should be kept in the FP service room. The service provider will obtain complete information on individual clients from a woman's card and copy all the required information to the FP register. This will help to compile and generate monthly FP service statistics reports. (see Annex D).

B. Long-Acting Removal Register

The Long-Acting Removal Register is used to record data for clients who have had long acting FP methods and who have returned for removal. The FP methods that are included for removal are implants (Implanon, Jadelle and levoplant) and IUCD. Data abstracted from women card and entered to the LAFP removal register by service providers.

13.4.4. Tally Sheets

A. FP Service Tally Sheet

It is used to summarize FP users by age and method to ease the reporting monthly. It is kept at FP service provision room and tallied immediately after transcribing the information on FP register.

B. FP Dispensed Count Tally

It is used to collect the total amount of contraceptives distributed by type of method. It is kept at FP service delivery point and tallied immediately after providing the FP method for clients. The information is aggregated and reported annually. At a health post level, the FP service and dispensed method count tally sheets are captured in a single format and used to summarize reportable data elements.

13.4.5. Reporting and Data Use

The ministry of health designed and is implementing HMIS that captures information from all government, private and other non-governmental agency health institutions to monitor health sector performance. Data need to be aggregated and reported from health facilities to the next level at monthly, quarterly and annual base. District Health information system (DHIS2) is a web-based platform used to aggregate statistical data collection, validation, analysis, management, and presentation. Currently all health facilities and administrative health units use DHIS2 for reporting, data analysis, performance

tracking and feedback provision. Similarly, FP data are reported and analyzed using this DHIS2 in monthly, quarterly and annual base.

13.4.6. FP Indicators

A set of high-priority indicators and operational targets will be objectively measured and used for monitoring and evaluation purposes to understand the scale and outcomes of implementation, to provide evidence-based guidance for actions/decisions and to improve accountability at all levels of the health system. The HMIS, while continuing to be vigorously strengthened, remains the main source of data for routine tracking of the performance of most of the interventions of the FP program

TABLE 9: FP INDICATOR MATRIX

No.	Indicator	Definition, calculation, and measurement level	FREQUENCY of measured?	Reporting level	Source of data	Data collection	Analysis & synthesis	Communication and use
1.1	Contraceptive acceptance rate (CAR)	Proportion of women of reproductive age (15-49 years) who are not pregnant and are accepting a modern contraceptive method (new and repeat acceptors) **Disaggregation By** type of acceptors: New, repeat; By Age: 10-14, 15 - 19, 20–24, 25–29, 30-49 years By methods: pills(OCPPOP,COC,ECP), Injectable (DMPA SC &IM),	Monthly	Health post/ Heath center /Clinic/ Hospital	FP register; Service delivery tally (for HP), RH register (clinics). Data should also be collected from abortion and integrated adolescent register	HMIS,	Data Quality Assurance; Triangulation of Data from different sources; Comparison of performance against benchmarks (Targets, baselines, standards and	Regular reporting, review by performance monitoring teams, regular review, ARM and other forums; midterm and final evaluation, Global reporting, share product related information in different platforms

		Implants(Jaddele, Implanon, levoplant, IUCD, Vasectomy ,Vasectomy, Tubal ligation (TL) and Disability status; yes/no and others **Numerator** Number of new and repeat acceptors **denominator** Total number of women of reproductive age (15-49 years) who are not pregnant					international and national commitments...)	
1.2	Immediate postpartum contraceptive acceptance rate (IPPCAR)	The proportion of women of reproductive age (15-49 years) who are accepting a modern contraceptive method immediately (0- 48 hrs.) after delivery **Disaggregation** by Age: 10-14, 15 - 19, 20–24, 25–29, 30-49 years, disability status(yes/no) &By methods: pills(POP, ,Implants (Jadelle, Implanon, Levoplant) Injectable (DMPA IM & SC), PP IUCD, Tuba Ligation, Others like, disability status Disaggregation by within 48 hr. and 48 hr. to 7 days Nominator Number of women who accepted a modern contraceptive method immediately	Monthly	Health post/ Heath center /Clinic/ Hospital	Delivery registers, PNC register, Service delivery tally (for HP), RH register (or primary private clinics	HMIS,	Data Quality Assurance; Triangulation of Data from different sources; Comparison of performance against benchmarks (Targets, baselines, standards and international and national commitments...)	Regular reporting, review by performance monitoring teams, regular review, ARM and other forums; midterm and final evaluation, Global reporting, share product related information in different platforms

		after delivery (0-48hours) Denominator Total Number of women who delivered in a facility						
1.3	Premature removal of LAFP methods	The number of mothers who used long term FP and removed before six months	Monthly	Heath center /Clinic/ Hospital	Long term removal register	HMIS	Data Quality Assurance; Triangulation of Data from different sources; Comparison of performance against benchmarks	Regular reporting, review by performance monitoring teams, regular review, ARM and other forums; midterm and final evaluation, Global reporting, share product related information in different platforms

MAJOR REPORTABLE DATA ELEMENTS AND INDICATORS FOR FP SERVICES

FP reportable Data elements

- FP New and repeat acceptors by method, age and disability status
- Immediate Postpartum acceptors by age, method and disability status
- Total number of premature removals of LAFP within 6-month insertion
- Total LAFP removal
- FP method dispensed
- Extended post-partum FP(48 hr-42days and 42 days to 1 year)
- Number or percent of women who delivered in the facility and receive FP counseling at: - ANC, L&D and PNC, (New versus repeat)-consider to catch up counseling from all units
- Introduce F/P integration register

FP indicators

- Contraceptive acceptance rate (Part of national Indicator)
- Long term FP coverage (Program Indicator)
- Immediate post-partum coverage (Program Indicator)
- Extended postpartum coverage (program indicator)
- Premature removal rate of LTFP (Program Indicator)

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ANNEXURES

Annex A: Couple-Years of Protection

A CYP is the estimated protection from pregnancy provided by contraceptive methods during a one-year period, based upon the volume of all contraceptives sold or distributed free of charge to clients during that period. CYP calculated by multiplying the quantity of each method distributed to clients by a conversion factor, to yield an estimate of the duration of contraceptive protection provided per unit of that method. The CYP for each method then summed for all methods to obtain a total CYP figure. CYP conversion factors are based on how a method is used, failure rates, wastage, and how many units of the method are typically needed to provide one year of contraceptive protection for a couple. The calculation considers that some methods, like condoms and oral contraceptives, for example, utilized incorrectly and then discarded, or that IUDs and implants removed before their life span realize CYP conversion factors updated on the 2022 measure evaluation indicators: USAID (https://www.usaid.gov/global-health/health-areas/family-planning/couple-years-protection-cyp)

Method	CYP Per Unit
Copper-T 380-A IUD	4.6 CYP per IUD inserted
Hormonal IUD (e.g. LNG-IUS)	4.8 per IUD inserted
3-year implant (e.g. Implanon, Implanon NXT, Levoplant)	2.5 CYP per implant
5-year implant (e.g. Jadelle)	3.8 CYP per implant
Emergency Contraception	0.05 CYP per dose (20 doses per CYP)
Fertility Awareness Methods	1.5 CYP per trained adopter
Standard Days Method	1.5 CYP per trained adopter
Lactational Amenorrhea Method (LAM)	.25 CYP per user (4 active users per CYP)
Sterilization Global	10CYPperprocedure
Combined Oral Contraceptives (COC) (blister packs of 28 pills)	0.0667 CYP per pack (15 cycles per CYP)
Progestin-only Pills (POP) (blister packs of 35 pills)	0.0833 CYP per pack (12 cycles per CYP)

Condoms (Male and Female)	0.00833 CYP per unit (120 units per CYP)
Diaphragm	1 CYP per diaphragm
Vaginal Foaming Tablets	0.00833 CYP per unit (120 units per CYP)
Depo Provera (DMPA) Injectable	0.25 CYP per dose (4 doses per CYP)
Noristerat (NET-En) Injectable	0.167 CYP per dose (6 doses per CYP)
Cyclofem Monthly Injectable	0.077 CYP per dose (13 doses per CYP)
Monthly Vaginal Ring/Patch	0.067 CYP per cycle (15 units per CYP)

Annex B: REDI-Family Planning Counseling Guide

REDI stands for rapport building, exploration, decision making, and implementing framework. It encourages open communication and less rigid counseling. REDI has different counseling approaches and techniques to counseling clients from different categories; new, repeat and satisfied and non-satisfied clients. Unlike other counseling frameworks, it also addresses whether and how the client will be able to carry out the decision he or she has made.

B-1: NEW CLIENT

R - Rapport-building	
Greet client with respect Assure confidentiality and privacy Ensure that there is nobody else listening to the talk and looking at the procedure Explain the need to talk about sensitive issues	●Welcome client; offer a seat; introduce yourself; ask client's name ● Affirm to the client that the subject would not be disclosed to any other person unless she/he wants to. ● Explain need to ask personal and sometimes sensitive questions about her sexual and reproductive health, which is helpful for making her visit successful.
E – Exploration	
Ask the reason for visit	● Previous FP method use, whether she has already decided on a method

Explore client's knowledge about FP method/s/ and fill the knowledge gaps	● Ask what she/he knows about the types of contraception and Provide information based on the gap about how to use, effectiveness, advantages, disadvantage and complications, protection against STI/HIV
Ask reproductive history and fertility plan	● Pregnancy history and outcomes, number and age of children, whether s/he wants more children, the nature of contraceptive protection desired (Duration, hormone/ non-hormone, etc.)
Explore client's circumstances and relationships	● Partner/spouse/family involvement and support for contraceptive use with particular emphasis on method(s) of interest; ability to communicate with the partner about FP decisions; other factors (socioeconomic) that may influence contraceptive use
Explore issues related to sexual life	● Questions/concerns/problems client has about sexual relations/practices; nature of sexual relationships (frequency, regularity) that may affect contraceptive choice and use whenever important
Ask about STI/HIV knowledge/ history and help to perceive risk	● Ask about knowledge, history of STI , any sign and symptoms on the client/partner, perceived risk of STI/HIV and explain the advantage of Dual protection to reduce the Risk,
Rule out pregnancy	● Ask about date of last birth, Breast Feeding practice, last menstrual period and menstrual pattern, history of unprotected sex, recent abortion/miscarriage etc. ● Ask whether client has any known or suspected health problems: Cardiovascular (including high blood pressure), liver,

Screen client for possible medical condition	reproductive cancer, bleeding/spotting between periods/after sex, severe anemia etc.
D - Decision making	
Help clients consider or remind the following before making decision: Encourage to make her/his own decision	● Eligibility for the method, if she can tolerate the side effects, STI/HIV risk protection and potential barriers ●Reconfirm it is her/his choice, confirm that the decision is voluntary
I – Implementation	
Explain the method Explain the procedure if there is one. Identify barriers to implement decision & develop strategies to overcome barriers Make a follow-up plan	●When to start, how to use and where to obtain the method, S/E and their Mx, Warning signs. ●Consider barriers like side effect, Partner relationship, cost and availability of method and deal with them like what to do with side effect, role of emergency contraceptive, options to switch, negotiation with partners, etc. and provide written information (if any) ● Timing of medical follow-up or resupply, ensure that client understood all information, remind the client to return or call whenever s/he has questions, concerns or problems

B-2: RETURNING CLIENT (WITH PROBLEM)

REDI counseling steps

R- Rapport-building	
Greet client with respect	Welcome client; offer a seat
E – Exploration	

Ask the purpose for visit Confirm correct method use Ask about satisfaction with Current method If there is dissatisfaction Ask about changes in circumstances and sexual life; new medical conditions	●Returning client with no problem or with problem Ask the client to describe how she is using the method ● Side effects (managing side effects or switching to another method) ● Incorrect method uses (discuss how to use method and backup method correctly) ● Suspected pregnancy (ask about client's and her partner's reaction to possible pregnancy, explain screening/ testing to be done); discuss method options if pregnancy screening/test are negative and options if result positive (e.g. ECP, if appropriate) ●Warning signs (explain screening/other exams, test and treatment to be done and referral as needed) ●Change in individual STI/HIV risk (help perceive her risk, dual method uses). ● Lack of partner or family support to use the method (discuss possible communication and other strategies that can help client continue with method) Ask if she has any health problems recently, if she has changed partner, concerns that she might be exposed to STI/HIV (ask about dual method use) since last visit;
D- Decision making	
Identify what decisions the client needs to confirm or make Encourage to make own decision	Continuing with current method, switching to another method, discontinuing FP method, STI/HIV risk reduction/dual protection, complying with treatment etc.

	Reconfirm her/his choice, confirm that the decision is voluntary
I – Implementation	
Help the client in implementing the decision: - Continue current method - Switch to another method - Discontinue the method Make a follow-up plan	● Help deal with the side effects ● Provide the information and skills (especially for condoms) needed for correct use of the Method ● Help to get services they need or refer (pre-conception or antenatal care ● For clients wanted removal of Implant or IUD, explain removal procedure and respond to question. ● Timing of medical follow up or resupply, ensure that client understood all information, remind to return or call whenever s/he has questions, concerns or problems

B-3: Returning client (SATISFIED)

REDI Counseling Guide

R - Rapport-building	
Greet client with respect	Welcome client; offer a seat
E – Exploration	
Ask the purpose for visit Ask about satisfaction with current method Confirm correct method use Ask if there are changes in circumstances and sexual life; if she develops any medical problem	● Ask what she/he feels about using the method ●Check if client has any questions /concerns /problems, especially regarding side effects ● Ask the client to describe how she is using the method(if it is administered by the client herself/himself) ● Ask if she has any problems regarding her health condition, if she has changed partner,

	concerns that she might be exposed to STI / HIV(ask about dual method use) since last visit;
D - Decision making	
Help client identify what services she needs during this return visit	●Re supply ●Regular well women visit ● Follow up visit etc.
I – Implementation	
Make a follow-up plan if, applicable	● Timing of medical follow up or resupply, ensure that client understood all information, remind to return or call whenever s/he has questions, concerns or problems

ANNEX C: Postpartum FP Counseling Guide during ANC Visit

Assess reproductive intentions	Ask about desired number of children, desire to space or limit births, desire for long-term FP, etc.
Determine any previous experiences with FP.	Ask whether she has had prior experience with FP methods, any problems, reasons for discontinuing, etc.
	Explore woman's knowledge about the return of fertility and the benefits of pregnancy spacing or limiting (as desired).
Provide information about benefits of healthy pregnancy spacing (or limiting, if desired).	Advise about the return of fertility postpartum and the risk of pregnancy. Advise how LAM and breastfeeding are different.
	Advise about the health, social and economic benefits of healthy pregnancy spacing (or limiting, if desired).
	Advice to ensure her health and the health of her baby (and family), she should wait at least 2 years after this birth before trying to get pregnant again.
	If she has any medical comorbidity, provide tailored counseling about timing of next pregnancy and preconception care.

Provide information about PPFP methods.	Based on woman's prior knowledge and interest, briefly explain the advantages, effectiveness, limitations and use of the following methods: • PPIUD • Implants • LAM • POPs • Postpartum tubal ligation (if permanent method is desired)
Determine interest in a particular FP method.	Ask whether she has a preference for a specific method, based on prior knowledge or the information provided.
Decision Making:	Help the woman to choose a method. Give woman additional information that she may need and answer any questions.
	Assess her knowledge about the selected method; provide additional information as needed.
Supports the woman's choice.	Acknowledge the woman's choice and advise her on the steps involved in providing her with her chosen method.
	Encourage the woman to ask questions and asks her to repeat key information.
Document selected PPFP method	***[String]**
	If the woman cannot arrive at a decision at this visit, asks her to plan for a follow-up discussion at her next visit. Inform her that she can bring partner/family member with her.
Care provider name:	

*String list (PPIUD, Implant, LAM, POPs BTL, opted for interval method initiation, opted for none)

Annex D: FP Register:-Ministry of Health, Ethiopia FP Register

Family Planning Register

Identification					Family Planning and contraceptive services																
Personal information					Registration			HIV Testing and Counselling								Clinical exam and contraceptive services provided			Follow-up and remark		
S.N	MRN	Name of Client	Age	Sex (M/F)	Reg. date (DD/MM/YY)	New acceptor (√)	Repeat acceptor(√)	HIV Test offered (√)	HIV Test performed (√)	HIV Test Result (P/N)	HIV specific Contra-ceptive counseling offered (√)	HIV Positive and linked to ART(√)	Target population Category write code	Td status checked (√)	Contraindication for IUCD (√)	Visit No.	Visit date (DD/MM/YY)	Contraceptive provided	Appointment date	Remark/Name &signature	
1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	
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New ▼ Rep

Use Abbreviation For (Col. 19)
Mc=Male condom
FeC=Female condom
OC=Oral contraceptive
Ec=Emergency Contraceptive
Inj=Injectable
Imp=Implant
IUCD=Intrautrine device
TL=Tubaligation
Vas=Vasectomy
Oth=Others

Targeted population category (Col. 14)
A. Female Commercial Sex workers
B. Long distance drivers
C. Mobile/Daily Laborers
D. Prisoners
E. OVC
F. Children of PLHIV
G. Partners of PLHIV
H. Other MARPS
I. General population

Annex E: Consent Form for Voluntary Surgical Contraception

በፍላጎት የተመሰረተ የቀዶ ህክምና የቤተሰብ እቅድ ተጠቃሚ ደንበኞች መተማመኛ ቅጽ እኔ ፊርማዬ ከዚህ በታች የሚገኝ ግለሰብ በቀዶ ህክምና የእርግዝና መከላከያ መጠቀም ስለምፈልግ ከዚህ በታች የተዘረዘሩትን ነጥቦች ተብራረተው ተነግረውኝ እና ተረድቼ ተቀብያለሁ። ይህዉም፡ 1ኛ. በቀዶ ህክምና ከሚደረገዉ የእርግዝና መከላከያ ውጭ ሌላ ጊዜያዊም ሆነ ቋሚ የእርግዝና መከላከያ ዘዴወችን በመጠቀም ቤተሰቤን ማቀድ እንደምችል መረጃ ተሰጥቶኛል፤ እኔም በሚገባ አዉቃለሁ። 2ኛ. ይህ በቀዶ ህክምና የሚፈጸም የቤተሰብ እቅድ ዘዴ ነው። እንደማንኛውም የቀዶ ህክምና ስርአት ደንብ ሰለሚደረግ አነዳንድ መጠነኛ የጎነዮሽ ችግሮች ሊከሰቱ እነድሚችሉ ተገልፆልኛል 3ኛ. ይህ የእርግዝና መከላከያ ዘዴ የዘለቄታ መሆኑ ሊታወቅ ይገባል። ሆኖም ግን ማንኛውም የቀዶ ህክምና መቶ በመቶ እርግዝናን የመከላከል ብቃትን ሊያስገኝ አይችልም፡፤ ለጥቂት ደንበኞችም ላይሰራ እንደሚችልና የቀዶ ህክምናው ውጤት ካልሰመረ ተጨማሪ ልጆች ሊኖረኝ እንደሚችል በሚገባ ተረድቻለው። (የወንዶች የዘር ፍሬ ማመላለሻን ማሰቋጠር በተለይ ለመጀመሪያ ሶስት ወራት ሙሉ በሙሉ ላይሰራ እነደሚችል ተገለጾልኝ ተረድቻለሁ።) 4ኛ.እኔዉ በራሴ ፍላጎት ያለምንም ተጽእኖና ግፊት የተቀበለኩ ስለሆነ ቀዶ ህክምናዉ ከመስራቱ በፊት በፈለግሁ ጊዚ ሃሳቤን በመለወጥ ድርጊቱን ላለመቀበል እንደምችል ተነግሮኛል፡ ሃሳቤንም በመለወጤ ምክያት የህክምና፤ የጤና እና ሌሎች አገልግሎቶችንና ጥቅሞች ከማግኘት እንደማይከለክለኝ ተነግሮኝ፡ መብቴም እነደሚጠበቅልኝ ተገንዝቤአለሁ። 5ኛ. ይህ የእርግዝና መከላከያ ዘዴ በግብረ ስጋ ግንኙነት ጊዜ የሚተላለፉ በሽታወችን (ኤ.ች.አይ.ቪን ጨምሮ) እንደማይከላከል ተረድቻለሁ።

የደንበኛው ስም፡---------- የሁኪሙ ስም፡----------

የደንበኛው ፊርማ፡---------- የሁኪሙ ፊርማ፡----------

ቀን---------- ቀን----------

Annex F: Referral form Ministry of Health; Ethiopia Referral form

Date _____

Client Full Name _____ age _____

sex_____ Medical record number_____

Address:

Region _____Woreda _____Kebele _____ Village_____

House number _____

Reason for Referral: ……..…….…

……..

Referred to _____ Referring Institution _____

Referred By: _____ Occupation_____

Signature_____ Date_____

…….

Referral Feedback

Date _____

Client Full Name _____

Age _____

Sex_____

Medical record number_____

Address:

Region _____Woreda _____Kebele _____

Village_____ House number _____

Name of Facility: _____

Feedback to (Institution) _____

Type of Service provided _____

Feedback_____

Follow up Plan_____

Appointment date: _____ _____

Service Provider: _____ Occupation_____

Signature_____ Date_____

Annex G: Internal Referral Forms

This internal referral form will be filled by the referring unit following client counseling. The referral form will be filled in two copies: one copy will be forwarded to the receiving unit (FP unit) and one copy will be retained in the referring unit.

Internal Referral/Linkage Form

Date ___/___/___

Name of the Client _____ MRN _____

Referring Unit _____

Referred to _____

Counseled on Family Planning: ☐ Yes ☐ No

Signature: _____

Annex H: Family Planning Quick Reference Chart: WHO 2015 Medical Eligibility Criteria

CONDITION	Sub-condition	COC	DMPA	Implants	Cu-IUD
Pregnancy		NA	NA	NA	
Breastfeeding	Less than 6 weeks postpartum				
	6 weeks to < 6 months postpartum				
	6 months postpartum or more				See i.
Postpartum and not breastfeeding VTE = venous thrombo-embolism	< 21days				
	< 21days with other risk factors for VTE*				
	≥ 21 to 42 days with other risk factors for VTE*				
	> 42 days				See i.
Postpartum and breastfeeding or not breastfeeding	< 48 hours or more than 4 weeks	See ii.	See ii.	See ii.	
	≥ 48 hours to less than 4 weeks				
	Puerperal sepsis				
Post-abortion	Immediate post-septic				
Smoking	Age ≥ 35 years, < 15 cigarettes/day				
	Age ≥ 35 years, ≥ 15 cigarettes/day				
Multiple risk factors for cardiovascular disease					
Hypertension BP = blood pressure	History of (where BP cannot be evaluated)				
	BP is controlled and can be evaluated				
	Elevated BP (systolic 140 - 159 or diastolic 90 - 99)				
	Elevated BP (systolic ≥ 160 or diastolic ≥ 100)				
	Vascular disease				
Deep venous thrombosis (DVT) and pulmonary embolism (PE)	History of DVT/PE				
	Acute DVT/PE				
	DVT/PE, established on anticoagulant therapy				
	Major surgery with prolonged immobilization				
Known thrombogenic mutations					
Ischemic heart disease (current or history of) **or stroke** (history of)				I C	
Known hyperlipidemias					
Complicated valvular heart disease					
Systemic lupus erythematosus	Positive or unknown antiphospholipid antibodies				
	Severe thrombocytopenia		I C		I C
	Immunosuppressive treatment				I C
Headaches	Non-migrainous (mild or severe)	I C			
	Migraine without aura (age < 35 years)	I C			
	Migraine without aura (age ≥ 35 years)	I C			
	Migraines with aura (at any age)		I C	I C	
Unexplained vaginal bleeding (prior to evaluation)					I C
Gestational trophoblastic disease	Regressing or undetectable β-hCG levels				
	Persistently elevated β-hCG levels or malignant disease				
Cancers	Cervical (awaiting treatment)				I C

	Endometrial				I	C
	Ovarian				I	C
Breast disease	Undiagnosed mass	**	**	**		
	Current cancer					
	Past w/ no evidence of current disease for 5 yrs					
Uterine distortion due to fibroids or anatomical abnormalities						
STIs/PID	Current purulent cervicitis, chlamydia, gonorrhea				I	C
	Vaginitis					
	Current pelvic inflammatory disease (PID)				I	C
	Other STIs (excluding HIV/hepatitis)					
	Increased risk of STIs					
	Very high individual risk of exposure to STIs				I	C
Pelvic tuberculosis					I	C
Diabetes	Nephropathy/retinopathy/neuropathy					
	Diabetes for > 20 years					
Symptomatic gall bladder disease (current or medically treated)						
Cholestasis (history of)	Related to pregnancy					
	Related to oral contraceptives					
Hepatitis	Acute or flare	I C				
	Chronic or client is a carrier					
Cirrhosis	Mild					
	Severe					
Liver tumors (hepatocellular adenoma and malignant hepatoma)						
High risk of HIV or HIV-infected (Stage 1 or 2)						
AIDS (HIV-infected Stage 3 or 4)	No antiretroviral therapy (ARV)	See iii.	See iii.	See iii.	I	C
	Improved to Stage 1 or 2 on ARV therapy					
	Not improved on ARV therapy				I	C
Drug interactions	Rifampicin or rifabutin					
	Anticonvulsant therapy***					

Category 1 There are no restrictions for use.
Category 2 Generally use; some follow-up may be needed.
Category 3 Usually not recommended; clinical judgment and continuing access to clinical services are required for use.
Category 4 The method should not be used.

A Not Applicable: Women who are pregnant do not require contraception. If these methods are accidentally initiated, no harm will result.

i. See condition "Postpartum and breastfeeding or not breastfeeding" instead.

ii. See condition "Breastfeeding" or condition "Postpartum and not breastfeeding" instead.

iii. Women who use methods other than IUDs can use them regardless of HIV stage or use of ART.

* Other risk factors for VTE include previous VTE, thrombophilia, immobility, transfusion at delivery, BMI > 30 kg/m2, postpartum hemorrhage, immediately post-caesarean delivery, pre-eclampsia, and smoking.

**Evaluation of an undiagnosed mass should be pursued as soon as possible.

*** Anticonvulsants include phenytoin, carbamazepine, barbiturates, primidone, topiramate, oxcarbazepine, and lamotrigine. Lamotrigine is a category 1 for implants.

Annex I:Internal Facility Report And Resupply Form

Internal Facility Report and Resupply Form (IFRR)

Name of Dispensing Unit: | Maximum Stock Level (ML)=

Reporting Period: From: | To:

Ser. No.	Product Description	Unit of Issue	Completed By Unit					Completed By Store		
			Beginning Balance	Quantity Received	Loss/ Adjustment	Ending Balance	Calculated Consumption	Maximum Quantity	Quantity Needed to Reach Max	Quantity to be Supplied
			A	B	C	D	E= A+B+/-C-D	F= E*2	G= F-D	H
	Family Planning (FP)									
1	Condoms (Male) Latex-180mm x 53mm	144								
2	Condoms (Female) (Need to be removerd)	144								
3	Etonogestrel - 68mg capsule (subdermal implant) with full insertion accessories kit (Implanon)	Kit								
4	IUCD (Intrauterine Contraceptive Device) - Long acting (CU380 A)	Each								
5	Levonorgestrel - 75mg/rod of 2 rods - Implant rods (subdermal) with sterile insertion trocar (Jadelle)	Each								
6	Levonorgestrel (D-Norgestrel) - 0.03mg - Tablets (Mini Pills)	Cycle								
7	Levonorgestrel (D-Norgestrel) - 0.75mg -Tablets (Emergency Contraceptive Pills)	Cycle								
8	Levonorgestrel (D-Norgestrel) + Ethinylestradiol + Ferrous Fumerate - (0.15mg + 0.03mg + 75mg) - tablets (COC)	Cycle								
9	Medroxyprogesterone Acetate - 150mg/ml in 1ml vial - Injection (Aqueous Suspension) with syringe 3ml, needle 21G (Depo Provera)	Vial								
10	PPIUCD-Postpartum Cu T 380 A IUD-weight of T frame 165 to 187 mg	each								

Remarks:

Completed by (Name, Date and Signature):

Completed by (Name, Date and Signature):

Approved by (Name, Date and Signature):

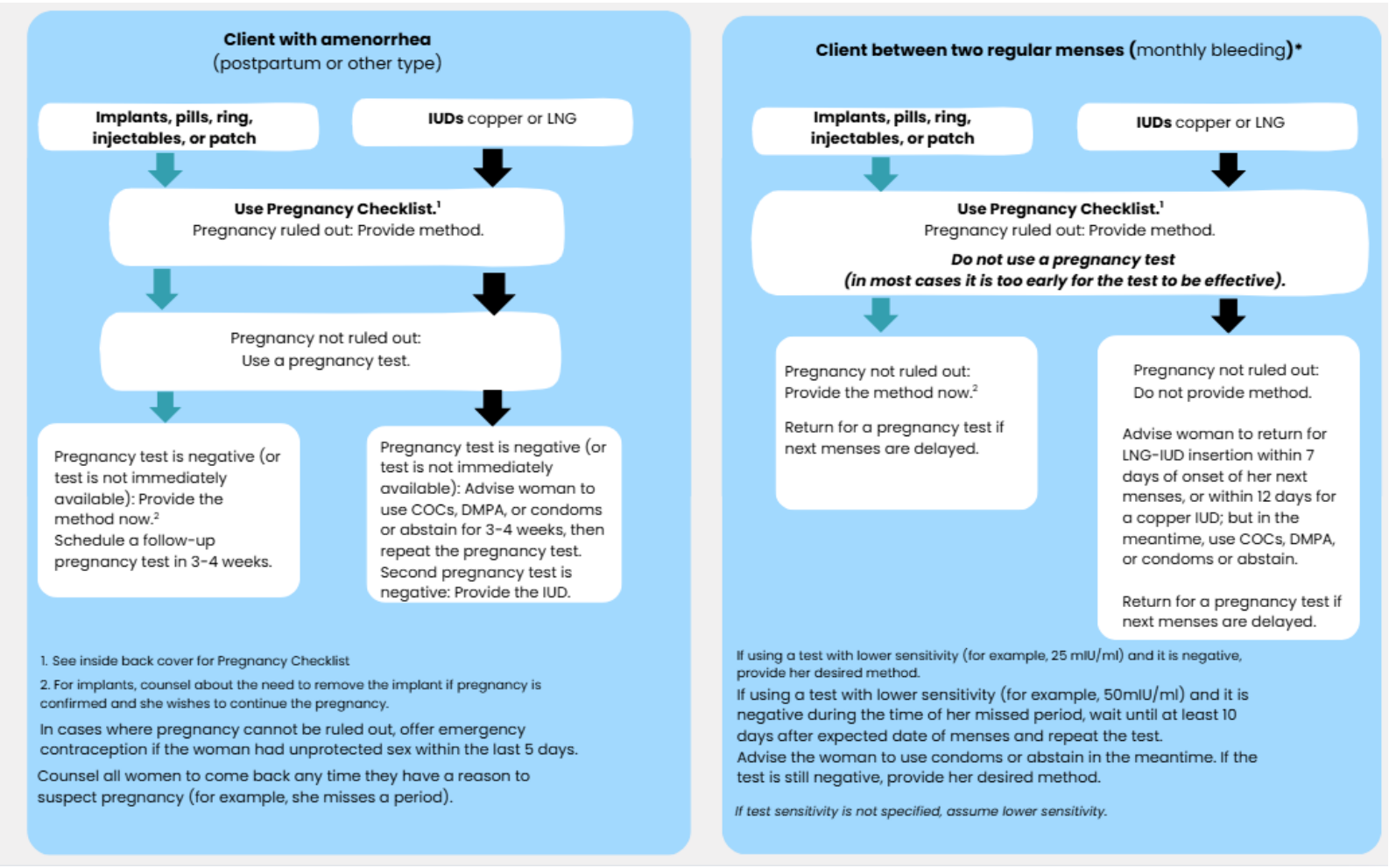
Annex J: Report and Requisition Form

Report and Requisition Form (RRF)													Page 3/5
Name of Health Facility:			Region:		Zone:			Woreda:		Facility Telephone No.:			
Supplying EPSS Branch:							Maximum Stock Level = 4 Months of Stock						
Reporting Period: From:		To:					Emergency Order Point = 0.5 Months of Stock						
			Report Part							Requisition Part			
						Ending Balance							
Ser. No.	Product Description	Unit of Issue	Beginning Balance	Quantity Received	Loss/ Adjustment	DU	Store	Calculated Consumption	Days Out of Stock	Maximum Stock Quantity	Quantity Needed to Reach Max	Quantity to Order	Remark on Quantity to be supplied (For EPSS Hubs Only)
			A	B	C	D	E	F= A+B +C - D-E	G	H=(120xF)/ (60-G)	I=H-E-D	J	K
	Family planning												
1	Condoms (Male) Latex- 180mm x 53mm												
2	Condoms (Female)												
3	Etonogestrel - 68mg capsule (subdermal implant) with full insertion accessories kit (**Implanon**)												
4	**IUCD** (Intrauterine Contraceptive Device) - Long acting (CU380 A)	Each											
5	Levonorgestrel - 75mg/rod of 2 rods - Implant rods (subdermal) with sterile insertion trocar (**Jadelle**)	Each											
6	Levonorgestrel (D-Norgestrel) - 0.03mg - Tablets (**Mini Pills**)	Cycle											
7	Levonorgestrel (D-Norgestrel) - 0.75mg -Tablets (**Emergency Contraceptive Pills**)	Cycle											
8	Levonorgestrel (D-Norgestrel) + Ethinylestradiol + Ferrous Fumerate - (0.15mg + 0.03mg + 75mg) - tablets (**COC**)	Cycle											
9	Medroxyprogesterone Acetate - 150mg/ml in 1ml vial - Injection (Aqueous Suspension) with syringe 3ml, needle 21G (**Depo Provera**)	Vial											
10	PPIUCD-Postpartum Cu T 380 A IUD-weight of T frame 165 to 187 mg	each											
Completed by:			Signature:			Date:			Mobile Number:				
Verified by:			Signature:			Date:			Mobile Number:				
Approved by:			Signature:			Date:			Mobile Number:				

Annex K: JOB AIDS

A. When And How to Use Pregnancy Check Lists and Pregnancy Test

Match your client's menstrual status and chosen contraceptive method with one of the options below and follow the instructions.



Pregnancy Checklist

Ask the client questions 1–6. As soon as the client answers "yes" to any question, stop and follow the instructions below

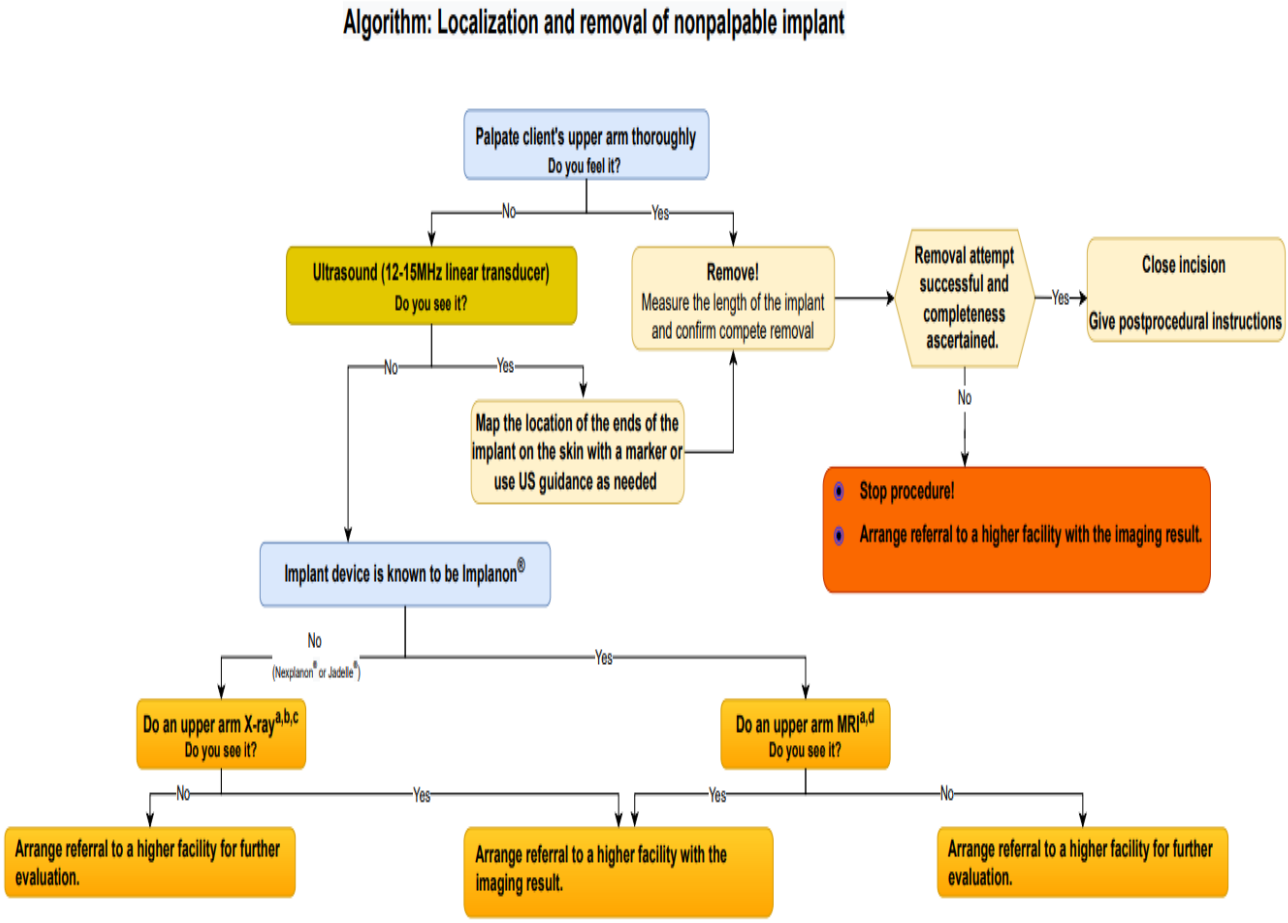
NO	Pregnancy Checklist Algorithm	YES
	1. Did your last monthly bleeding start within the past 7 days*?	
	2. Have you abstained from sexual intercourse since your last monthly bleeding, abortion, or miscarriage?	
	3. Have you been using a reliable contraceptive method consistently and correctly since your last monthly bleeding, abortion, or miscarriage?	
	4. Did you have a baby in the last 4 weeks?	
	5. Have you had a baby less than 6 months ago, are you fully or nearly-fully breastfeeding, and have you had no monthly bleeding since then?	
	6. Have you had a miscarriage or abortion in the past 7 days*?	

***If the client is planning to use a copper-bearing IUD, the 7-day window is expanded to 12 days.**

If the client answered NO to all of the questions, pregnancy cannot be ruled out using the checklist. Rule out pregnancy by other means.

If the client answered YES to at least one of the questions, you can be reasonably sure she is not pregnant.

B. Non-Palpable Implant Evaluation and Removal Algorithm



[a]Communicate clearly with the radiologist what you are looking for.

[b]Request two views (BOTH AP and lateral views).

[c]For Jadelle: request imaging to be set at 50–55 kV and 4–5 mA with an exposure time of 0.03 seconds.

[d]The fat-suppression technique should be avoided and the interslice gap should be ≤ 2mm.

C. Missed IUD Evaluation and Removal Algorithm

